

HEALTH CARE SERVICES AGREEMENT

BETWEEN

KAISER FOUNDATION HEALTH PLAN OF WASHINGTON

AND

RADIANTCARE PHYSICIANS, PLLC



ARTICLE 1 DEFINITIONS

- 1.1 Business Continuity Plan
- 1.2 Clean Claim
- 1.3 Covered Services
- 1.4 Kaiser Payer
- 1.5 Member
- 1.6 Member Cost Share
- 1.7 Official(s)
- 1.8 Other Payer
- 1.9 Payer
- 1.10 Plan
- 1.11 Policies
- 1.12 Proprietary Information
- 1.13 Services

ARTICLE 2 PROVIDER'S RESPONSIBILITIES

- 2.1 Provision of Services
- 2.2 Non-Discrimination
- 2.3 Licensure and Credentials
- 2.4 Subcontracts
- 2.5 Information
- 2.6 Policies
- 2.7 Law
- 2.8 Business Continuity Plan

ARTICLE 3 KFHPWA'S RESPONSIBILITIES

ARTICLE 4 BILLING AND PAYMENT

- 4.1 Compensation
- 4.2 Claims
- 4.3 Prompt Payment
- 4.4 Member Billing
- 4.5 Member Hold Harmless
- 4.6 Coordination of Benefits
- 4.7 Liens and Third-Party Claims
- 4.8 Review and Audit
- 4.9 Adverse Events

ARTICLE 5 TERM AND TERMINATION

- 5.1 Term
- 5.2 Termination Without Cause
- 5.3 Termination With Cause
- 5.4 Immediate Termination
- 5.5 Effect of Termination
- 5.6 Suspension of Participation

ARTICLE 6 DISPUTE RESOLUTION

- 6.1 Member Grievance
- 6.2 Informal Review Dispute Resolution Process
- 6.3 Formal Review Dispute Resolution Process

ARTICLE 7 RECORDS AND CONFIDENTIALITY

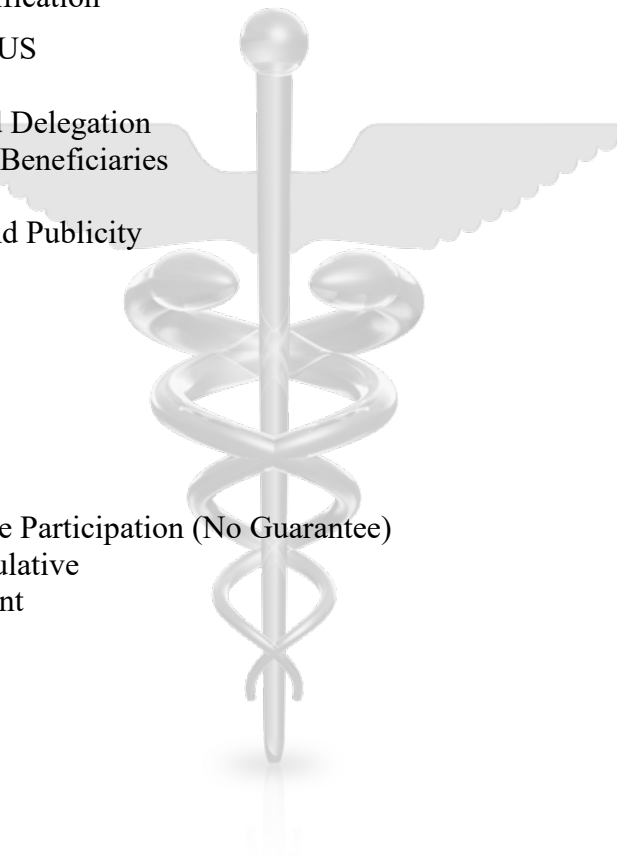
- 7.1 Maintenance of Records
- 7.2 Access to Records
- 7.3 Security

ARTICLE 8 RELATIONSHIP OF PARTIES

- 8.1 Communication with Members
- 8.2 Independent Contractor
- 8.3 Government Contractor
- 8.4 Proprietary Information
- 8.5 Insurance
- 8.6 Mutual Indemnification

ARTICLE 9 MISCELLANEOUS

- 9.1 Notice
- 9.2 Assignment and Delegation
- 9.3 No Third-Party Beneficiaries
- 9.4 Force Majeure
- 9.5 Use of Name and Publicity
- 9.6 Governing Law
- 9.7 Severability
- 9.8 Waiver
- 9.9 Amendment
- 9.10 Interpretation
- 9.11 Counterparts
- 9.12 No Exclusivity
- 9.13 Network/Service Participation (No Guarantee)
- 9.14 Remedies Cumulative
- 9.15 Entire Agreement
- 9.16 Attorney Fees



HEALTH CARE SERVICES AGREEMENT

This Health Care Services Agreement (“**Agreement**”) is entered into between **Kaiser Foundation Health Plan of Washington**, a nonprofit corporation organized and existing under the laws of the State of Washington (hereinafter referred to as “**KFHPWA**”) and **RadiantCare Physicians, PLLC** (hereinafter referred to as “**Provider**”). This Agreement is effective for Services (as defined below) rendered on and after 15th day of February, 2025 (“**Effective Date**”).

WHEREAS, KFHPWA provides or arranges for health care services to Members (as defined below); and

WHEREAS, KFHPWA has retained Washington Permanente Medical Group, PC (“**WPMG**”) to provide or arrange for professional services to Members; and together, KFHPWA and WPMG cooperate to form the Kaiser Permanente Medical Care Program in the Washington Region; and

WHEREAS, KFHPWA shall be the corporation responsible for the obligations of a licensed health maintenance organization with respect to Members with individual or group coverage issued by KFHPWA and regulated by the applicable state insurance commissioner; and

WHEREAS, KFHPWA desires to enter into an arrangement for Provider to render Services to Members;

NOW THEREFORE, the parties agree as follows:

ARTICLE 1 DEFINITIONS

When used in this Agreement, these capitalized terms shall have the following meanings.

- 1.1 “**Business Continuity Plan**” means a collection of procedures and information that is developed, compiled, and maintained in readiness for use in the event of an emergency or disaster.
- 1.2 “**Clean Claim**” means a claim that (1) is completed with all data elements required by Payer and applicable law, (2) if submitted electronically, is submitted using standard code sets as required by law, and (3) has no defect or error that prevents timely adjudication.
- 1.3 “**Covered Services**” means the health care services that a Member is entitled to receive and that are properly authorized under the terms and conditions of a Plan.
- 1.4 “**Kaiser Payer**” means: (i) any regional Permanente Medical Group, and (ii) KFHPWA and any company controlling, controlled by, or under common or shared control with KFHPWA (including Kaiser Permanente Insurance Company and wholly owned subsidiaries of KFHPWA) when a Member of such a company under any circumstance (such as, but not limited to, vacations, temporary work assignments, and direct referrals for specialty care), seeks care from Provider under a Plan regulated by a state insurance or managed care commissioner. With respect to applicable state laws regarding health maintenance organizations and health care service contractors, KFHPWA is responsible for the conduct of such companies and their compliance with the terms of this Agreement.
- 1.5 “**Member**” means an individual entitled to receive Covered Services.
- 1.6 “**Member Cost Share**” means a copayment, deductible, or coinsurance amount payable by a Member for Covered Services pursuant to the Member’s Plan.

- 1.7 **“Official(s)”** mean(s) (1) individuals who represent, in an official capacity, a local, state or federal government agency or regulatory body with jurisdiction over any KFHPWA entity or Provider; (2) representatives of any accreditation agency or organization (such as the National Committee for Quality Assurance) or a peer review body or other peer review or professional organization applicable to a KFHPWA entity or Provider; (3) such other officials entitled by law or pursuant to government contracts with KFHPWA (including Medicare, Federal Employees Health Benefits Program, Medicaid, and TriCare, as applicable) to monitor health care services provided to Members; and (4) the designees of any of the above.
- 1.8 **“Other Payer”** means any public or private entity – including employers, insurers, third party administrators, labor unions, trusts, associations, and other organizations, persons, and entities – that is responsible for funding a Plan and that enters into an administrative or management service arrangement with a Kaiser Payer to administer its Plan.
- 1.9 **“Payer”** means a Kaiser Payer or Other Payer.
- 1.10 **“Plan”** means a plan of health benefits administered, issued, sponsored, or underwritten by either a Kaiser Payer or an Other Payer, as set forth in the Member’s summary plan description, coverage agreement, evidence of coverage, certificate of coverage, or other written coverage document.
- 1.11 **“Policies”** means the policies and procedures of Kaiser Payer and Other Payers that relate to this Agreement including: (1) quality assurance, quality improvement and management and patient safety; (2) utilization management and referral and authorization processes; (3) pre-admission testing guidelines; (4) claims submission, review, and payment; (5) member grievances; (6) provider credentialing; and (7) electronic transmission of data. Policies include those policies and procedures set forth in manuals, bulletins, and newsletters, whether made available by postal mail, electronic mail, web site, or other media.
- 1.12 **“Proprietary Information”** means all information, whether prepared by Provider, KFHPWA, Plan, Payer, or their representatives, relating to each such entity or the development, execution or performance of this Agreement whether furnished or obtained prior to or after the Effective Date. Proprietary Information includes pricing, financial information, rate schedules, and Member information collected by Plans and Payers not otherwise set forth in medical records; provided, however, the following shall not constitute Proprietary Information: (1) information known prior to receipt from the other party; (2) information previously available to the public prior to receipt; (3) Proprietary Information that subsequently becomes available to the public through no fault or omission on the part of the party receiving the Proprietary Information; and (4) information independently developed without use of the disclosing party’s Proprietary Information.
- 1.13 **“Services”** means those services, supplies, and other resources described in Exhibit “Compensation and Services” that Provider customarily provides to its general patient population.

ARTICLE 2 PROVIDER'S RESPONSIBILITIES

- 2.1 **Provision of Services** Provider shall make available to and provide Members with Services, along with any related facilities, equipment, personnel, or other resources necessary to provide Services, according to generally accepted standards of practice. Services shall be readily available and accessible to Members and provided in a prompt and efficient manner without unreasonable delays. Provider shall assume responsibility for the manner in which Services are rendered and the obligation to exercise independent judgment in providing Services to Members.
- 2.2 **Non-Discrimination** Provider shall provide Services to Members without discrimination on the basis of race, ethnicity, color, gender, sex, creed, religion, national origin, age, health status, physical or mental disability, genetic information, veteran's status, marital status, sexual orientation, gender identity, income, source of payment, participation in a government program, evidence of insurability, medical condition, claims experience, receipt of health care, conditions arising out of acts of domestic violence, status as a Member, or any other status protected by applicable law; and Provider shall ensure that Provider's facilities and equipment are accessible to persons with disabilities. Provider shall make Services available to all classes of Members, in the same manner, in accordance with the same standards, and with the same availability, as with respect to Provider's other patients.
- 2.3 **Licensure and Credentials** Provider represents and warrants that it (together with its employees, agents, and contractors providing Services) shall throughout the term of this Agreement: possess and maintain, without restriction, all applicable federal, state and local licenses, permits, certificates, approvals and authorizations required to render Services to Members (including, if applicable, medical staff membership and clinical privileges at facilities designated by KFHPWA); comply with KFHPWA credentialing requirements, as described in Policies; remain accredited or certified by the organizations designated by KFHPWA; be enrolled in, participate in, meet coverage conditions for and, where applicable under Centers for Medicare and Medicaid Services rules, be certified by the Medicare and Medicaid programs; and not (1) be sanctioned by, debarred from, suspended from, precluded from, excluded from, opted out of, or otherwise ineligible for any government program, including Medicare or Medicaid, or (2) be identified on any government list of sanctioned, suspended, excluded or otherwise ineligible entities and individuals, including lists maintained by the Department of Health and Human Services, General Services Administration, Office of Inspector General and Office of Foreign Assets Control. Provider shall comply with the standards of any organization accrediting KFHPWA as they apply to Services rendered to Members.
- 2.4 **Subcontracts** If Provider provides Services indirectly, by arranging for the provision of Services to a Member by any individual or entity not a bona fide employee of Provider ("Subcontractor"), Provider shall enter into a written contract with such Subcontractor prior to the provision of Services to Members by the Subcontractor. Such contract shall require the Subcontractor to comply with the same terms applicable to Provider under this Agreement. Upon request, Provider shall promptly provide access to and copies of all contracts with Subcontractors. Unless otherwise required by law or arranged with KFHPWA, Provider shall retain responsibility for paying its Subcontractors (in accordance with any law applicable to subcontractors of health care service contractors), and such Subcontractors shall not seek reimbursement from or have any recourse against a Payer for Subcontractor's services provided to Members.

2.5 **Information** Provider shall notify KFHPWA Promptly (as defined below) upon Provider's knowledge of any of the following involving Provider or any employee, agent or Subcontractor providing Services: (1) the revocation, suspension, restriction, or expiration of any license, permit, certification, approval, authorization, medical staff membership or clinical privilege required to render Services to Members or of any accreditation or certification specified in Section 2.3 (Licensure and Credentials); (2) sanction by, debarment from, suspension from, preclusion from, exclusion from, opt out from, or other ineligibility for any government program, including Medicare or Medicaid, or identification on any government list of sanctioned, suspended, excluded or otherwise ineligible entities and individuals, including lists maintained by the Department of Health and Human Services, General Services Administration, Office of Inspector General and Office of Foreign Assets Control; (3) the submission of a formal report to a professional board, licensing agency, or the National Practitioner Data Bank of adverse credentialing or peer review action; (4) a material change in credentialing or privilege status; (5) any change in operations that is reasonably likely to materially affect the manner in which Services are provided to Members; (6) any unusual occurrence involving a Member that is reported or required to be reported to a regulatory body or an accreditation organization; (7) any change in legal status, tax identification number, or NPI or Medicare or Medicaid number; (8) any material change in ownership, control, name, or location; (9) the initiation of any legal action, accreditation organization action, or regulatory or governmental action that is likely to materially and adversely affect Provider's or a Subcontractor's ability to perform its obligations under this Agreement; (10) any professional liability claim or other legal action filed or asserted by a Member against Provider or an employee, agent, or Subcontractor of Provider; (11) any event or circumstance that is reasonably likely to materially impair the ability to provide Services to Members; (12) bankruptcy, dissolution or receivership of Provider, or an assignment by Provider for the benefit of creditors; and (13) any recall, market withdrawal, or safety alert notice issued by any federal or state government agency, or the distributor or manufacturer of the item, regarding any item delivered to a Member under this Agreement. However, Provider shall not be required to waive legal privilege in order to comply with subsections (3), (6), (9), and (10) of this Section, provided that Provider shall notify KFHPWA pursuant to these subsections to the extent Provider can do so without waiving legal privilege. For purposes of this Section "Promptly" means within two (2) business days for subsections (1), (2), (3), (6), and (13) and within ten (10) business days for the other subsections in this Section. With respect to subsection (13), at the request of KFHPWA, Provider shall provide specific information regarding the item identified in the notice, the Member(s) to whom the item was delivered, and the date of delivery.

2.6 **Policies** Provider shall cooperate and comply with the then-current Policies of which Provider knows or reasonably should have known. Policies may be modified by Payers from time to time, but no Policy change shall be retroactive with respect to Provider without the express consent of Provider. In the event of any inconsistency between a Policy and this Agreement, this Agreement shall prevail.

A. **Cooperation and Compliance** Without limiting the foregoing, Provider shall cooperate and comply with Payers' quality assurance, quality improvement, utilization management and patient safety activities (including pre-service, concurrent, and retrospective reviews), case management and disease management services, preauthorization steps required for Covered Services, member and provider grievance and appeals processes, pharmaceutical formularies, claims submission and payment procedures.

B. **Member Issues** Provider shall investigate and respond promptly to issues regarding quality of care, patient safety, accessibility, and other complaints (including Member complaints) related to Provider's Services to Members. Provider shall use best efforts to promptly address and remedy any unsatisfactory condition related to the care of Members, as determined by KFHPWA or an Official. All benefit decisions regarding Covered Services are reserved to the Plan, and Provider shall refer Members who have benefit inquiries or disputes regarding what constitutes Covered Services to the Plan for response and resolution.

2.7 **Law** Provider shall comply with all laws, rules, and regulations applicable to Provider and Services under this Agreement, including those set forth in Exhibits attached hereto.

2.8 **Business Continuity Plan** Provider shall maintain a Business Continuity Plan that addresses the methods to minimize the impact of events causing an interruption of Services to ensure consistent performance of Services hereunder. Provider agrees to maintain and routinely test a comprehensive Business Continuity Plan to ensure appropriate and timely recovery of the ability to provide Services to Members during times of business interruption. Provider's Business Continuity Plan must address methods to minimize the impact of an event on the health and safety of Members and to ensure consistent quality performance and service from Provider. Upon request, Provider shall promptly provide KFHPWA a copy of the Business Continuity Plan for review. In addition, Provider shall incorporate required changes and other feedback received from KFHPWA regarding its Business Continuity Plan. A Business Continuity Plan should be reviewed and updated as required by operational needs, but in no case less frequently than once per year. Revisions should address changes to technology, functions, procedures, and/or personnel that could impact the integrity and viability of the recovery plan. If Provider has knowledge of a potential service disruption that may affect KFHPWA or Members, Provider shall notify KFHPWA and the applicable Plan(s) promptly. In addition, Provider may be asked to provide each Plan with a 24/7 emergency contact. If Provider is determined to be critical to KFHPWA's operations, Provider may be asked to participate in KFHPWA's business continuity plan exercise drills, and Provider shall make commercially reasonable efforts to cooperate.

ARTICLE 3 KFHPWA'S RESPONSIBILITIES

KFHPWA shall ensure that Payers (1) provide Members with sufficient information to permit Provider to determine Plan benefits, Member eligibility for Plan benefits, and other necessary Plan information and (2) appropriately designate Provider in directories.

ARTICLE 4 BILLING AND PAYMENT

4.1 **Compensation** Compensation to Provider for Services that are Covered Services to Members shall be paid at the lesser of Provider's billed charges or the rates set forth in Exhibit "Compensation and Services," less any Member Cost Share. Provider accepts such amount as payment in full for all Services that are Covered Services and acknowledges that compliance with Section 2.3 (Licensure and Credentials) is a precondition to payment. Pursuant to the procedures in Section 4.4 (Member Billing), Provider shall also accept from the Member payment at the rates set forth in Exhibit "Compensation and Services" as payment in full for all Services that are not Covered Services.

- 4.2 **Claims** Provider shall submit to the applicable Payer a Clean Claim for Covered Services rendered to Members. Provider shall maximize the use of electronic claims rather than paper claims. Provider agrees that Payers shall not be obligated to make payments for billings received more than three hundred sixty-five (365) days from (1) the date of service or (2), when a Payer is the secondary payer, from the date of receipt by Payer of the primary payer's explanation of benefits. This requirement may be waived, in the discretion of the applicable Payer, in the event Provider provides written notice and appropriate evidence to the Payer of extraordinary circumstances outside the control of Provider that resulted in the delayed submission. If Provider does not timely submit a claim, or does not timely dispute any alleged underpayment, Provider's claim for payment shall be deemed waived, and Provider shall not seek payment from the Plan, Payer, KFHPWA, or Member.
- 4.3 **Prompt Payment** Within thirty (30) days of receipt of a Clean Claim from Provider, KFHPWA shall (1) cause Kaiser Payer to pay Provider for Services that are Covered Services rendered to Members of Kaiser Payer and (2) to the extent permitted by law, notify Other Payers to authorize payment to Provider for Services that are Covered Services rendered to an Other Payer's Members; provided, that such period shall be extended as reasonably necessary where benefits must be coordinated with another health plan. KFHPWA is not responsible for the promptness or ultimate payment of claims for services rendered to Members of any Plan other than KFHPWA. KFHPWA shall, however, reasonably assist Provider in collecting delinquent or underpayments from Payers.
- 4.4 **Member Billing** Provider shall bill Members for applicable Member Cost Share. Provider may bill Members, subject to Section 4.1 (Compensation), for Services that are not Covered Services if prior to the services being rendered, the Member agreed in writing to pay for such services after being advised the services may not be Covered Services. Provider may not bill Members for Services that are Covered Services where payment is denied based upon utilization management decisions of Payers or where Provider failed to comply with the terms or conditions of this Agreement. Provider may bill or charge individuals who were not Members at the time that services were rendered.
- 4.5 **Member Hold Harmless** In no event, including nonpayment by or insolvency of KFHPWA or a Payer, or breach of this Agreement, shall Provider bill; charge; collect a deposit from; seek compensation, reimbursement, or remuneration from; impose surcharges; or have any recourse against any Member, person acting on the Member's behalf, or any person other than the responsible Payer for Covered Services provided under this Agreement, except for Member Cost Share payment. The terms of this Section shall be construed to be for the benefit of Members and shall supersede any oral or written agreement to the contrary now existing or hereafter entered into between Provider and the Member or persons acting on the Member's behalf.
- 4.6 **Coordination of Benefits** When a Payer accessing the rates under this Agreement is the primary payer under applicable coordination of benefit principles, that Payer shall pay for Covered Services in accordance with this Agreement. When such Payer is secondary under applicable coordination of benefit principles, Provider is owed for Covered Services the lesser of (1) an amount equal to the Member's liability as determined by the primary payer or (2) the difference between the applicable rates under this Agreement less the amounts paid by the primary payer, unless otherwise required by law. However, if Provider provides services for an employment-related injury or illness compensable under workers' compensation or employment liability law, Provider shall look to the applicable workers' compensation carrier or responsible employer for compensation and shall not be entitled to additional payment under this Agreement for such services paid under workers' compensation or employment liability law.

- 4.7 **Liens and Third-Party Claims** In instances of third-party liability claims (involving, for example, liability carriers), Provider shall accept the amount payable under this Agreement as payment in full. Provider shall not, directly or indirectly through assignment or otherwise, assert any lien claim, subrogation claim, or any other claim against a Member or any other person or organization against which a Member may hold a potential claim for personal injury, or against the proceeds of a Member's personal injury recovery, based on Services provided to a Member for an injury or illness allegedly caused by a third party.
- 4.8 **Review and Audit** Unless otherwise required by law, KFHPWA, Provider, Plan, Payer, and their authorized agents may review and audit claims to verify the appropriateness and amount of payment so long as such review and/or audit takes place within two (2) years of the date of payment or denial (or such longer period as may be set forth in an Other Payer's Plan or as required by a government program), except in cases of fraud, for which no time limit applies. Unless otherwise required by law, a refund or payment to correct an undisputed overpayment or underpayment shall be due upon demand, and in the case of an overpayment, failure to make such payment within thirty (30) days of receipt of a demand shall entitle the Payer to offset, recoup or deduct the amount owed from any other amounts owed to Provider. To the maximum extent permitted by law, Payers are authorized to offset and recoup the amount of any debt owed by Provider to Payer against any debt or money owed to Provider.
- 4.9 **Adverse Events** Consistent with rules and other directives promulgated by the Centers for Medicare and Medicaid Services (CMS), consistent with Washington State law, and as set forth in the Policies, KFHPWA shall not reimburse Provider, and Provider shall not bill Payer, for treatment and/or services arising from or related to preventable "Adverse Events," as defined under Washington law or non-medically necessary early elective deliveries, as defined in the Bree Collaborative Obstetrics Report (August 2012), that occur at Provider's facility. Provider shall waive any copayments and other costs and reverse any charges associated with services related to preventable Adverse Events that occur at Provider's facility and shall take steps to ensure that Members are not required to pay Provider for care related to a preventable Adverse Event at Provider's facility. Provider also agrees to maintain a policy that prohibits Provider billing for preventable Adverse Events, and non-medically necessary early elective deliveries, provide a copy of such policy to Payer upon request, and provide information to Payer upon request regarding the number and type of preventable Adverse Events that occur at Provider's facility. Payer shall make requests for reports on preventable Adverse Events under the auspices of KFHPWA Coordinated Quality Improvement Program and shall seek to maintain the confidentiality of such reports consistent with state and federal law.

ARTICLE 5 TERM AND TERMINATION

- 5.1 **Term**
This Agreement shall begin on the Effective Date and, subject to earlier termination as described in this Article 5, shall continue for an initial term of two (2) years expiring on February 14, 2027. At the expiration of this initial term, this Agreement shall automatically renew for one (1) year periods thereafter unless any party gives the other prior written notice of non-renewal at least one hundred twenty (120) days prior to the end of the initial term or a renewal term, as applicable.
- 5.2 **Termination Without Cause** Notwithstanding Section 5.1 (Term), this Agreement may be terminated by a party without cause at any time by giving the other party prior written notice of no less than 120 days.

- 5.3 **Termination With Cause** This Agreement may be terminated upon written notice by a party for material breach by the other party, provided that (1) such non-breaching party gives at least sixty (60) days' prior written notice of termination to the party in breach and a description of the other party's breach, and (2) such material breach is not cured during the sixty (60)-day notice period; subject, however, to Section 5.4 (Immediate Termination). Material breach by Provider shall include Provider's failure to comply with Section 2.3 (Licensure and Credentials) or Section 2.6 (Policies).
- 5.4 **Immediate Termination** Any of the following events shall, at the sole discretion of KFHPWA, result in the immediate termination or suspension of this Agreement, upon written notice to Provider: (1) the withdrawal, debarment, suspension, expiration, restriction or non-renewal of any federal, state or local license, certificate, approval or authorization of Provider required to render Services; (2) the bankruptcy, dissolution or receivership of Provider, or an assignment by Provider for the benefit of creditors; (3) the loss or material limitation of Provider's liability insurance required under this Agreement; (4) a determination by KFHPWA that Provider's continued participation in this Agreement could result in imminent and substantial harm to Members; (5) sanction by, debarment from, suspension from, preclusion from, exclusion from, opt out of, or other ineligibility for Provider's participation in any government program including Medicare or Medicaid, or identification of Provider on any government list of sanctioned, suspended, excluded or otherwise ineligible entities and individuals, including lists maintained by the Department of Health and Human Services, General Services Administration, Office of Inspector General and Office of Foreign Assets Control; (6) the conviction of Provider of any crime; (7) the revocation, suspension, restriction or expiration of Provider's accreditation or credentialing status or the cessation of Provider's operations; or (8) a change of ownership or control of Provider.
- 5.5 Effect of Termination
- A. **Cooperation in Transfer of Members** Upon termination or expiration of this Agreement, Provider shall cooperate in the timely and efficient transfer of Members to other facilities and/or providers designated by KFHPWA or the Plan. KFHPWA shall arrange for timely notification of Members of the termination or expiration of this Agreement.
- B. **Survival** Termination or expiration shall not affect those rights, powers, remedies, liabilities, and obligations that accrued or arose before termination or expiration. The obligations under Section 4.1 (Compensation), Section 4.2 (Claims), Section 4.5 (Member Hold Harmless), Section 4.6 (Coordination of Benefits), Section 4.8 (Review and Audit), Section 5.5 (Effect of Termination), Article 6 (Dispute Resolution), Article 7 (Records and Confidentiality), Section 8.4 (Proprietary Information), the tail coverage requirement of Section 8.5A (Provider's Insurance), Section 9.2 (Assignment and Delegation), Section 9.3 (No Third Party Beneficiaries), and Section 9.5 (Use of Name and Publicity), and any provisions of this Agreement expressly stated to survive termination or expiration or that, by their nature, are intended to survive or cannot be fully performed prior to expiration or termination, shall (i) survive termination or expiration of this Agreement, regardless of the circumstances of termination or expiration, and (ii) supersede any oral or written contrary agreement now existing or hereafter entered into by the parties as they relate to Services provided to Members during the term of this Agreement.

C. **Continuation of Care** Upon termination or expiration of this Agreement, Provider shall continue to provide (and Payer shall continue to compensate Provider for) Services pursuant to this Agreement and the Policies to Members who are under the care of Provider at the time of termination or expiration until the later to occur of the discharge or transfer of the Member in accordance with an appropriate professional standard of care or such time period as may be prescribed by applicable law.

5.6 **Suspension of Participation** Any of the following events may result in KFHPWA's immediate suspension, until cured to KFHPWA's satisfaction, of a practitioner or site of Provider providing Services under this Agreement from participation in this Agreement and the KFHPWA provider network (without terminating or suspending the Agreement), upon written notice to Provider: (1) the withdrawal, debarment, suspension, expiration, restriction or non-renewal of any federal, state or local license, certificate, approval, or authorization of such practitioner or site required to render Services; (2) the failure of the practitioner or site to comply with KFHPWA's credentialing requirements, and if applicable to the practitioner, the loss or restriction of practitioner's medical staff membership or clinical privileges; (3) sanction by, debarment from, suspension from, preclusion from, exclusion from, opt out of, or other ineligibility for Provider's participation in any government program including Medicare or Medicaid, or identification of Provider on any government list of sanctioned, suspended, excluded or otherwise ineligible entities and individuals, including lists maintained by the Department of Health and Human Services, General Services Administration, Office of Inspector General and Office of Foreign Assets Control; (4) the conviction of the practitioner or site of any crime; (5) failure of the practitioner or site to comply with law or Policies following a reasonable request for compliance by KFHPWA; or (6) a determination by KFHPWA that the practitioner's or site's continued participation in this Agreement could result in imminent and substantial harm to Members.

ARTICLE 6 DISPUTE RESOLUTION

- 6.1 **Member Grievance** Provider agrees to (1) cooperate with and participate in Member appeal, grievance, and external review procedures; (2) provide the Plan with the information necessary to conduct Member appeal, grievance, and external review procedures; and (3) abide by the Plan's decisions regarding Member appeals, grievances, and reviews.
- 6.2 **Informal Review Dispute Resolution Process** Each Plan shall provide an internal mechanism whereby Provider may raise issues, concerns, controversies, or claims arising from or related to this Agreement. Specifically, each Plan shall maintain an informal review process pursuant to which Provider may seek to resolve disputes arising from this Agreement. This informal review process shall be exhausted before Provider may pursue further action against a Plan or Payer. Provider shall exhaust the applicable informal review process described below prior to invoking the formal review process.

- A. **Adverse Benefit Determination Appeal** The appeal process is available to seek reconsideration of an adverse benefit determination which per RCW 48.43.005 , or any successor thereto, means a denial, reduction, or termination of, or a failure to provide or make payment, in whole or in part, for a benefit, including a denial, reduction, termination, or failure to provide or make payment that is based on a determination of a Member's eligibility to participate in a Plan, and including, with respect to group health plans, a denial, reduction, or termination of, or a failure to provide or make payment, in whole or in part, for a benefit resulting from the application of any utilization review, as well as a failure to cover an item or service for which benefits are otherwise provided because it is determined to be experimental or investigational or not medically necessary or appropriate. For standard (non-expedited) appeals, Provider may supply supporting statements for a Member's appeal or with Member permission may file the appeal on a Member's behalf, or Provider may request an expedited appeal if the Member's life, health, or ability to regain maximum function would be jeopardized by following the standard appeal process. More information about appeals is provided in the "Caring For Our Members" section of the Policies. Instructions for the appeal process, including KFHPWA contact information, are located in the "Referrals and Clinical Review" section of the Policies.
- B. **Provider Payment** If Provider disagrees with the final disposition of a claim, Provider must contact the Provider Assistance Unit to initiate the first-level review process. If Provider disagrees with the outcome of the first-level review, Provider may initiate a request for reconsideration by completing the claims second-level reconsideration form. Instructions for both processes, including KFHPWA contact information, are provided in the "Billing and Claims" section of the Policies.
- C. **Contract Termination**: To request review of KFHPWA's decision to terminate this Agreement, you must submit to your contract manager any additional information for KFHPWA to consider within thirty (30) days of the termination notice. KFHPWA contact information is provided in the termination notice.
- D. **Credentialing** Eligible medical professionals have access to a dispute resolution process for non-reportable events when denied credentialing or recredentialing for administrative reasons, such as failure to meet credentialing criteria, privileging criteria, or both, and a fair hearing and appeal process for reportable events when they are denied initial credentialing, recredentialing or privileging by KFHPWA's Credentialing Committee, Board of Directors, or an authorized committee of the Board of Directors, or subject of an official action or recommendation for the reduction, restriction, suspension, revocation, termination, denial, or failure to renew membership, privileges, or both. Information about these processes is provided in the "Working With Kaiser Permanente" section of the Policies.
- E. **Regional Fraud Control Unit (RFCU)** If Provider disagrees with the disposition of a claims audit conducted by RFCU that resulted in payment recovery, Provider shall submit a request for reconsideration to the regional compliance office as set forth in the RFCU repayment notice.

- F. **Other Alleged KFHPWA Acts or Omissions** Provider shall contact their assigned KFHPWA contract manager to initiate informal, good faith discussions between Provider, or its designee, and the KFHPWA [Director of Provider Contracting] and/or other appropriate KFHPWA staff within thirty (30) days of KFHPWA's receipt of such request. Documentation of any mutually agreed resolution shall be forwarded in writing by KFHPWA to Provider within five (5) days of the final meeting.

6.3 **Formal Review Dispute Resolution Process** The parties intend to resolve expeditiously and efficiently any dispute, controversy, or claim arising from or related to this Agreement ("**Dispute**") that is not resolved pursuant to Section 6.2 (Informal Review Dispute Resolution Process) by promptly invoking the following formal dispute resolution procedures:

- A. **Scope of Disputes Subject to This Process** Provider may initiate the process set forth in this section only to address a dispute arising out of this Agreement.
- B. **Formal Review by KFHPWA** If after exhausting the informal review process Provider is dissatisfied with the outcome, Provider or an authorized representative must utilize the formal process set forth in this Section.
- C. **Injunction** Notwithstanding Section 6.3A, the parties agree that Disputes involving certain breaches of the Agreement, including a breach of the confidentiality obligations, would cause irreparable injury to the injured party that could not be compensated adequately in damages. The parties further agree that such injured party shall be entitled (in addition to any other remedies or damages) to remedies of injunction, specific performance, or restraining orders, which remedies do not require arbitration as a prerequisite. In such instance, the parties agree that the injured party may seek remedies of injunction, specific performance, or restraining orders in a civil court of competent jurisdiction.
1. To pursue a dispute, Provider or an authorized representative must submit a written request for review to KFHPWA's Vice President, Contracting, Network Operations, Strategy, hereinafter referred to as Vice President, or his/her authorized designee, at the mailing address in Article 9.1 no later than thirty (30) calendar days following disposition of the informal review of KFHPWA's act and/or omission on which the dispute is based. The written request must set forth a clear and concise statement of the issue, the specific provisions of this Agreement that are alleged to have been violated, and a statement of the remedy requested by Provider.
 2. Provider or authorized representative may request Vice President or authorized designee meet with Provider or authorized representative regarding the dispute. Such request for a meeting must be made at the time the dispute is presented to Vice President or authorized designee, and Provider or authorized representative must otherwise meet the requirements of this section.
 3. In the absence of a request for a meeting under this section, Vice President or authorized designee may elect to engage in discussions with Provider or authorized representative regarding the dispute, or the Vice President or authorized designee may issue a response without such discussions.

4. Vice President or authorized designee will provide Provider or authorized representative with a written response to the dispute no later than the later of a) twenty (20) days following the completion of discussions (if Provider or authorized representative requests a meeting or the Vice President or authorized designee elects to engage in discussions) or after meeting with Provider or authorized representative, as provided in this section, or b) thirty (30) days after the Vice President's or authorized designee's receipt of the request for review (if Provider or authorized representative does not request a meeting and the Vice President or authorized designee elects not to engage in discussions). This response will set forth the Vice President's or authorized designee's final determination regarding the dispute and, if appropriate, the remedy to be awarded. If KFHPWA fails to grant or reject a dispute within thirty (30) days of submission of such dispute, Provider or authorized representative may proceed as if the dispute had been rejected.
5. Billing Disputes. KFHPWA will render a decision on billing disputes within sixty (60) days of its receipt of the complaint.

D. **Mediation** If Provider or authorized representative has first exhausted the process of formal review, and appeal if applicable, set forth above and is not satisfied with the final determination of the Vice President or authorized designee, Provider or authorized representative may request nonbinding mediation under this section. Provider or authorized representative may also request mediation under this section if KFHPWA failed to grant or reject the dispute within thirty (30) days from the submission of such dispute. Such mediation will not preclude Provider or authorized representative from pursuing judicial remedies.

1. To request mediation, Provider, or authorized representative, must submit a written request for mediation to Vice President or authorized designee (at the mailing address in Article 9.1) no later than twenty (20) days following receipt of the Vice President's or authorized designee's final determination under this section. Provider's or authorized representative's request for mediation will constitute a promise to pay one-half the cost of the mediation service and Provider or representative shall be liable for payment of one-half the cost of the mediation service.
2. The Vice President or authorized designee, will forward Provider's or authorized representative's request for mediation to an Approved Mediator, as defined below. The Approved Mediator will be available to conduct the mediation session no later than thirty (30) days following his/her selection.
3. The "Approved Mediator" is a mediator who is mutually agreeable to KFHPWA and Provider who has been selected from an independent mediation service made available to Provider under the dispute resolution process provided for in this Agreement. The Approved Mediator shall schedule and conduct the mediation no later than thirty (30) days following his/her selection, unless both KFHPWA and Provider or authorized representative, agree to a later date.

4. If the mediation process results in an agreement between KFHPWA and Provider or authorized representative that resolves the dispute, the terms of that agreement shall be reduced to writing and implemented by KFHPWA and Provider or authorized representative. If the mediation process does not result in an agreement between KFHPWA and Provider or authorized representative that resolves the dispute, completion of the mediation process terminates KFHPWA's dispute resolution process.

ARTICLE 7 RECORDS AND CONFIDENTIALITY

- 7.1 **Maintenance of Records** Provider shall maintain its financial, accounting, administrative, and patient medical records in a current, detailed, organized, and comprehensive manner in accordance with prudent industry practice, applicable laws, and accreditation standards. All Member medical records shall be treated by Provider as confidential in accordance with applicable laws. Provider shall maintain these records with respect to a Member for the longer of ten (10) years after the last date Services were provided to the Member, or the period required by law. Should Provider experience a disclosure of Member information impermissible under state privacy laws or the privacy rules of the Health Insurance Portability and Accountability Act, Provider shall notify KFHPWA of the nature of the disclosure and the identity of the Members involved and shall take all steps required by law and reasonable business practice to remedy, mitigate and report the disclosure.
- 7.2 **Access to Records** In accordance with applicable law, KFHPWA, the applicable Plan and Payer, their authorized agents, and Officials shall have access to (including electronic access where practicable), a right to photocopy, and upon reasonable notice, a right to perform site visits related to, all financial, accounting, administrative, and patient medical records (including electronic images that are part of medical records) pertaining to Members for the purpose of meeting legal, regulatory and accreditation requirements applicable to the Plans; assessing quality of care and patient safety, conducting medical evaluations and audits; performing utilization management functions; verifying the accuracy of amounts paid or payable to Provider; and other functions of a Plan. In addition, Provider shall supply periodic reports pertaining to Services provided to Members as the parties may from time to time, agree or as otherwise required for Plans to meet their legal and accreditation requirements. Photocopies of medical records and other files, reports, books, and records shall be without charge to KFHPWA, Payers and Plans unless otherwise required by law. Provider agrees to allow access to or supply copies of such records, as requested, within fourteen (14) days of the receipt of a request, where practicable, and in no event later than the date required by any applicable law or Official.
- 7.3 **Security** If Services are rendered remotely, including through telemedicine, Provider shall establish and maintain physical, data, and information security policies, procedures and practices consistent with generally accepted industry standards to prevent the unauthorized access or use of confidential information and Member communications information and data, including medical records and Protected Health Information (as defined in the Health Insurance Portability and Accountability Act, Health Information Technology for Economic and Clinical Health Act, and their implementing regulations, all as amended from time to time).

ARTICLE 8 RELATIONSHIP OF PARTIES

- 8.1 **Communication with Members** Nothing in this Agreement shall be construed to limit Provider's ability to freely communicate with a Member or the Member's authorized representative about the Member's treatment options.
- 8.2 **Independent Contractor** Provider is an independent contractor to KFHPWA. Nothing in this Agreement is intended to create, nor shall it be construed to create, between KFHPWA and Provider a relationship of principal, agent, employee, partnership, joint venture, or association. KFHPWA and Provider have no authorization to enter into any contracts, assume any obligations, or make any warranties or representations on behalf of the other. No individual through whom Provider renders Services shall be entitled to or shall receive from KFHPWA compensation for employment, employee welfare and pension benefits, fringe benefits, or workers' compensation, life or disability insurance or any other benefits of employment, in connection with providing Services. Provider represents and warrants that it shall be solely responsible for the timely payment of wages, proper classification of its workers, workers' compensation insurance, employee benefits, any payroll-related taxes, and any other employment-related liability for its workers, and for all legally required tax withholding for itself and its employees.
- 8.3 **Government Contractor** As organizations with federal government contracts, KFHPWA and Kaiser Payers are subject to various federal laws, regulations, and executive orders (such as regarding equal opportunity and affirmative action), which may apply to KFHPWA's subcontractors. Notice of such laws, regulations, and executive orders is provided in Exhibit "Federal Program Compliance" in the Section on the Federal Employee Health Benefits Program. If Provider is not otherwise subject to compliance with such laws, regulations and executive orders, their reference in this Agreement shall not be deemed to impose such requirements upon Provider.
- 8.4 **Proprietary Information** Provider and KFHPWA agree the Proprietary Information of the other is the exclusive property of the other and that they have no right, title, or interest in the Proprietary Information of the other. Provider and KFHPWA agree to keep Proprietary Information strictly confidential and agree not to disclose any Proprietary Information to any third party, except (1) to Officials having jurisdiction, (2) as required in legal proceedings or government administrative proceedings or as required by law or the requirements of an accrediting or regulatory agency or government contract, (3) in the case of KFHPWA's disclosure, to Members, Plans, Payers, affiliates in the Kaiser Permanente Medical Care Program, consultants and vendors under contract with KFHPWA, or (4) as otherwise directed by the other party in writing; provided, however, with respect to (1) and (2), prior to any such compelled disclosure, the receiving party shall (to the extent allowed under applicable law) notify the disclosing party and cooperate fully with the disclosing party in protecting against any such disclosure and, if applicable, obtaining a protective order narrowing the scope of such disclosure and use of the Proprietary Information.
- 8.5 **Insurance**
- A. **Provider's Insurance** Provider at Provider's sole cost and expense shall procure and maintain such policies of insurance (or programs of self-insurance that are adequately funded according to sound actuarial principles). Such insurance coverage shall cover the acts and omissions of Provider as well as those of Provider's agents and employees at any sites where Services are provided to Members. Insurance coverages shall be carried at minimum levels required from time to time by KFHPWA, but in no event less than

- B. ONE MILLION DOLLARS (\$1,000,000) per claim / THREE MILLION DOLLARS (\$3,000,000) annual aggregate for comprehensive general liability insurance.
- C. ONE MILLION DOLLARS (\$1,000,000) per claim / THREE MILLION DOLLARS (\$3,000,000) annual aggregate for professional liability insurance / Provider may obtain one or more claims-made policies to fulfill its obligations under this Section so long as Provider obtains any extended reporting endorsements (tail coverage) for such policies as may be necessary to provide continuous coverage without interruption throughout the term of this Agreement and for at least three (3) years following termination or expiration of this Agreement. Provider shall maintain a fidelity bond or crime policy, in amounts no less than ONE MILLION DOLLARS (\$1,000,000) per claim / ONE MILLION DOLLARS (\$1,000,000) annual aggregate, covering theft and dishonesty related to Members' and others' assets at all sites (if any) to the extent Services are provided to Members, including (if applicable) in Members' homes or custodial care locations.
- D. Provider shall also procure and maintain workers' compensation insurance and unemployment insurance to the extent required by law.
- E. **ADDITIONAL REQUIREMENTS FOR PROVIDERS INSURANCE**
 - 1. Commercial insurance policies must be issued by insurance carriers with an A.M. Best rating of A-, VIII, or better, or equivalent.
 - 2. KFHPWA must be named as an additional insured where it is commercially allowable. Coverage for the additional insureds shall apply on a primary and non-contributory basis irrespective of any other insurance, whether collectible or not.
 - 3. Upon request, Provider will provide KFHPWA with certificates of insurance evidencing compliance with all insurance provisions noted above.
 - 4. Provider agrees to make best efforts to provide to KFHPWA at least thirty (30) days' advance written notice, and in any event shall provide written notice as soon as reasonably practicable, of any cancellation or material modification of these policies.
- F. **KFHPWA Insurance** KFHPWA, at its sole cost and expense, shall procure and maintain such policies of general and professional liability and other insurance (or programs of self-insurance that are adequately funded according to sound actuarial principles) as shall be necessary to insure itself and its employees against any claim or claims for damages arising by reason of personal injuries or death occasioned directly or indirectly in connection with the performance of any service by KFHPWA under this Agreement.

8.6 **Mutual Indemnification**

- A. **Provider** Provider shall indemnify and hold harmless, KFHPWA and all other persons or organizations cooperating in the conduct of the health care program commonly known as the Kaiser Permanente Medical Care Program, and their officers, directors, shareholders, partners, employees and agents from and against all demands, claims, losses, damages, liability, costs expenses, judgments or obligations, actions or causes of action to the extent arising from or in connection with the negligent acts or omissions or willful misconduct of Provider, and/or its officers, directors, shareholders, partners, employees and agents.

- B. **KFHPWA** KFHPWA shall indemnify and hold harmless Provider and its officers, directors, shareholders, partners, employees and agents from and against all demands, claims, losses, damages (including reasonable attorney fees), liability, costs, expenses, judgments, or obligations, actions or causes of action to the extent arising from or in connection with the negligent acts or omissions or willful misconduct of KFHPWA and/or its officers, directors, shareholders, partners, employees and agents.

ARTICLE 9 MISCELLANEOUS

- 9.1 **Notice** Any notices to be given under this Agreement shall be in writing, signed by an authorized signatory, and shall be deemed given upon receipt if sent to the addresses listed below as follows: (1) transmitted by electronic mail to the recipient's email address; (2) sent by United States Postal Service or a commercial delivery service, postage prepaid with proof of delivery (such as return receipt requested); or (3) sent through an electronic signature process with proof of acknowledgment by the recipient; or (4) personally delivered. Any party may change its address for notice purposes by following the process described in this Section.

KFHPWA

Vice President, Contracting, Network Operations, Strategy
Kaiser Foundation Health Plan of Washington
PO Box 34262
Seattle, WA 98124-1262
Telephone: (206) 988-2000
Email: prnotification@kp.org

PROVIDER

RadiantCare Physicians, PLLC
Gregory Allen, MD
Managing Member
4525 3rd Ave SE Ste 100
Lacey, WA 98503-1010
Telephone: (360) 412-8960
Facsimile: (360) 412-8970
Email: allen@radiantcare.com

- 9.2 **Assignment and Delegation** Except as otherwise provided in this Agreement, Provider shall not assign this Agreement or subcontract or delegate any of its duties and obligations under this Agreement without the prior written consent of KFHPWA. Any attempted assignment, delegation or subcontracting without such consent shall not release Provider from liability to KFHPWA for acts or omissions of Provider or the intended assignee, delegate or subcontractor and shall allow KFHPWA to terminate this Agreement for cause and seek additional available remedies. Any change of ownership or control of Provider or sale of substantially all of its assets shall be deemed an assignment. Any succession or assignment shall not relieve or otherwise affect the liability of Provider, which shall remain jointly and severally liable with the successor or assignee. Upon notice to Provider, KFHPWA may assign, subcontract, or delegate this Agreement or duties hereunder to an entity that is part of the Kaiser Permanente Medical Care Program.

- 9.3 **No Third-Party Beneficiaries** With the exception of Section 4.5 (Member Hold Harmless), and except as otherwise provided with respect to KFHPWA and Payers, nothing in this Agreement shall be construed to give any person other than Provider or KFHPWA any benefits, rights, or remedies.
- 9.4 **Force Majeure** Any delay or failure in the performance by a party of its obligations under this Agreement shall be excused if and to the extent caused by the occurrence of a force majeure. For the purposes of this Agreement, a force majeure means any extraordinary circumstance that is beyond the reasonable control of the party claiming force majeure and that prevents or impedes the due performance of the Agreement by that party, including war, acts of terrorism, riots or civil commotions, acts of government, fires, floods, hurricanes, earthquakes or other natural disasters or adverse weather conditions, widespread infections, epidemics or pandemics, embargoes or boycotts, and labor disputes or disturbances. The party claiming force majeure shall give written notice to the other party as soon as practicable (including the date of occurrence and the anticipated duration) and shall use its best efforts to minimize the potential adverse effects to the other party; and the other party shall be entitled to take any necessary measures or actions, including temporarily making alternative arrangements to fulfill the obligations under the Agreement until the party claiming force majeure can resume performance of the Agreement.
- 9.5 **Use of Name and Publicity** Each party reserves to itself the right to, and the control of the use of, its own names, symbols, trademarks and service marks, presently existing or hereafter established, and no party shall use another party's names, symbols, trademarks or service marks in any advertising or promotional materials or communication of any type or otherwise without the latter party's prior written consent; provided, however, KFHPWA and Plans may use the name, address and telephone number of Provider in lists of contracting providers and other marketing materials. KFHPWA and Provider shall consult with each other regarding any issue relating to this Agreement or to the delivery of Covered Services to Members under this Agreement that gives rise to media interest or public relations concern and shall cooperate in developing any statements or press releases in connection with any such issue.
- 9.6 **Governing Law** Except as preempted by federal law, this Agreement shall be governed by the laws of the state in which Provider is located, without application of conflict of law's provisions of such state.
- 9.7 **Severability** Any determination that any provision of this Agreement or any application thereof is invalid, illegal, or unenforceable shall not affect the validity, legality, and enforceability of such provision in any other instance, or the validity, legality, or enforceability of any other provision of this Agreement.
- 9.8 **Waiver** A failure of any party to exercise any provision of this Agreement shall not be deemed a waiver. To be effective, any waiver of any provision of this Agreement shall be in writing and signed by the party against whom the waiver is sought to be enforced. Any such waiver shall not operate or be construed as a waiver of any other provision of this Agreement or a future waiver of the same provision.

- 9.9 **Amendment** No changes, amendments, or alterations to this Agreement, including its Exhibits, shall be effective unless in a writing signed by the parties. Notwithstanding any other provision of the Agreement, if either party reasonably determines that a modification of this Agreement is necessary to cause it to be in compliance with applicable law or the requirements of an accrediting or regulatory agency or government contract, that party shall give the other party written notice of the proposed modification, the justification for the modification, and the date on which it is to go into effect, which shall not be less than thirty (30) days following the date of the notice (unless a shorter period of time is required by law or the requirements of an accrediting or regulatory agency or government contract). The modification shall go into effect on that date. The party providing notice shall consider any objection made by the other party concerning the proposed modification during the notice period.
- 9.10 **Interpretation** The captions or headings contained in this Agreement are included for purposes of convenience only and shall not affect in any way the meaning or interpretation of any of the terms or provisions of this Agreement. This Agreement shall be interpreted according to its fair intent and not for or against any one party on the basis of whether such party drafted the Agreement. All references to “including” or “include(s)” shall mean “including, without limitation” and “include(s) without limitation,” respectively. The omission of a particular example or the inclusion of any examples shall not be construed to broaden or limit the effect of the language. Any reference to a statute, regulation, government agency or program, regulatory body, accreditation standard or accreditation organization refers to the statute, regulation, government agency or program, regulatory body, accreditation standard or accreditation organization as amended from time to time, and to any successor statute, regulation, government agency or program, regulatory body, accreditation standard or accreditation organization. References to Provider shall include references to Provider, any employees, or agents of Provider, and any of Provider’s assignees, delegates, or Subcontractors, and their employees or agents.
- 9.11 **Counterparts** This Agreement and any amendments may be executed in separate counterparts, none of which need contain the signatures of all parties, and each of which, when so executed, shall be deemed an original and all together constitute and be one and the same instrument. Facsimile or electronic signatures shall be as valid as original signatures.
- 9.12 **No Exclusivity** This is not an exclusive agreement. Provider and KFHPWA may enter into similar agreements with other parties.
- 9.13 **Network/Service Participation (No Guarantee)** There is no representation, warranty, guarantee, or covenant that any minimum volume or value or type of business will be referred to Provider. Provider does not have a right to participate in any particular Plan, line of business, product, or network of a Plan. KFHPWA reserves the right to determine Provider’s Services and participation in one or more of the KFHPWA Payers’ Provider networks and products. KFHPWA has sole discretion to implement or discontinue products and provider networks and to decide whether to include Provider in written or electronic materials related to its products and provider networks.
- 9.14 **Remedies Cumulative** The rights and remedies of this Agreement shall not be exclusive and are in addition to any other rights and remedies provided by law.

9.15 **Entire Agreement** This Agreement, including its Exhibits, constitutes the complete and sole understanding of the parties regarding the subject hereof and supersedes all prior or contemporaneous oral or written representations, communications, proposals, or agreements not expressly included herein. This Agreement (including its Exhibits) may not be contradicted or varied by evidence of prior, contemporaneous, or subsequent oral representations, communications, proposals, agreements, prior course of dealings or discussions of the parties. There are no oral agreements among the parties. Provider has not relied on any data, financial analysis, reports, notes, proposals, conclusions, or projections, whether made orally or in writing, made by KFHPWA or any of its representatives, agents, employees, or advisors, in connection with negotiation, acceptance, execution or delivery of this Agreement by Provider.

9.17 **Attorney Fees** Except as provided for Section 8.6 (Mutual Indemnification), in the event of any action for relief, declaratory or otherwise, which arises between the parties, the prevailing party shall recover reasonable attorney's fees and costs.

IN WITNESS WHEREOF, the parties have caused this Agreement to be executed by their respective duly authorized representatives.

KAISER FOUNDATION HEALTH PLAN OF WASHINGTON

A Nonprofit Corporation, State Of Washington

Signed by:
By: Alacia H Broussard
6A4AF7E99C1743A...
Alacia Broussard
Title: Vice President, Contracting, Network Operations and Strategy (CNO&S)
Date: 2/19/2025 | 9:28 AM PST

RadiantCare Physicians, PLLC

By: Gregory Allen
Gregory Allen, MD
Title: Managing Partner
Date: 2/7/25

LIST OF EXHIBITS

Provider Network Participation

Compensation & Services

Federal Program Compliance

National Committee For Quality Assurance (“NCQA”) Required Provisions

Washington State Compliance



PROVIDER NETWORK PARTICIPATION EXHIBIT

This Exhibit is required by the Washington State Office of the Insurance Commissioner to identify the in-network participation of providers rendering health care services in the State of Washington. Provider participates in the following commercial networks in the State of Washington, which networks may be updated from time to time with notice to Provider.

Kaiser Foundation Health Plan of Washington

<u>Networks</u>	<u>Limited Services</u>
-----------------	-------------------------

Core	☐
------	--------------------------

Boeing HMO Network	☐
--------------------	--------------------------

SoundChoice	☐
-------------	--------------------------

Connect	☐
---------	--------------------------

Kaiser Foundation Health Plan of Washington Options, Inc.

<u>Networks</u>	<u>Limited Services</u>
-----------------	-------------------------

Options	☐
---------	--------------------------

Access PPO	☐
------------	--------------------------

Elect PPO	☐
-----------	--------------------------

Summit PPO	☐
------------	--------------------------

The applicable compensation associated with these networks is set forth in the Compensation Addendum and Compensation & Services Exhibit(s) of the Agreement

NETWORK PARTICIPATION COMPENSATION ADDENDUM

This Addendum is required by the Washington State Office of the Insurance Commissioner to identify the in-network participation of providers rendering health care services in the State of Washington. Provider participates in the following commercial networks in the State of Washington, which networks may be updated from time to time with notice to Provider.

The compensation related to Members in **COMPENSATION & SERVICES EXHIBIT(S)** sets forth the financial terms associated with the following commercial networks. These financial terms apply uniformly to all commercial networks and do not differ by network.

Kaiser Foundation Health Plan of Washington

Networks

- Core
- Boeing HMO Network
- SoundChoice
- Connect

Kaiser Foundation Health Plan of Washington Options, Inc.

Networks

- Options
- Access PPO
- Elect PPO
- Summit PPO

Tier 1: Preferred In Network ☒

Tier 2: In Network ☐

Summit PPO is a tiered provider network within the six-county service area of King, Snohomish, Pierce, Kitsap, Thurston, and Spokane. It identifies and groups physicians, healthcare professionals, ancillary providers, facilities, and hospitals into specific tiers in which different Coinsurance and Copayment amounts apply as a means of managing cost, utilization, and quality. KFHPWA does not use the Summit PPO tiered network to limit access to any certain categories of providers or facilities.

Kaiser Foundation Health Plan of Washington and Kaiser Foundation Health Plan of Washington Options, Inc. will provide notice to any affected providers of proposed changes to the tier criteria sixty (60) days before implementing. The notice will explain the methodology and data, if any, used for particular providers and include information on provider appeal rights. Kaiser Foundation Health Plan of Washington and Kaiser Foundation Health Plan of Washington Options, Inc. will make its physician/facility cost profiles available to providers under a tiered network, including the written criteria by which the provider's performance is measured.

PROFESSIONAL, ANCILLARY, AND FACILITY CRITERIA AND ASSOCIATED METRICS	
Tier 1-Preferred In-Network status: • Total cost of care risk arrangement in place (with both upside and downside financial risk) - The metrics utilized are total cost of care per member per month statistics and other utilization metrics at or exceeding a benchmark metric of WPMG’s performance as similarly measured. • Pharmacy mail order performance at or exceeding benchmark metric of WPMG delivery performance as similarly measured. • Provider integration into medical management process as measured against WPMG’s medical management performance.	Tier 2- In-Network status: • Physicians, healthcare professionals, ancillary providers and facilities who have a direct contract and are currently participating in the Access PPO network and service area. Exceptions may occur in the cases like our leased networks, First Choice Health Network (FCHN), WholeHealth Living (WHL), Optum RX (Pharmacy PBM).
HOSPITAL CRITERIA AND ASSOCIATED METRICS	
Tier 1-Preferred In-Network status: • Hospital is owned and operated by KFHPWA or its affiliates. • KFHPWA/ WPMG has physicians on-site at contracted hospital to align care management and care coordination efforts. • Hospital is sole hospital in geographic area and required for network adequacy.	Tier 2- In-Network status: • Hospital has a direct contract with KFHPWA or KFHPWAO.

EXHIBIT COMPENSATION & SERVICES

Purpose

KFHPWA is an organization that provides managed health care services to Members either directly or through contracts with Providers.

The purpose of this Agreement is to make available covered Services to certain Members according to the terms and conditions set forth above and below. Provider hereby authorizes and appoints KFHPWA to offer Provider's professional Services to all groups of employees and individuals covered under Medical Coverage Agreements entered into or designated by KFHPWA.



COMPENSATION

Professional Financial Terms

The following terms shall govern KFHPWA's payment for Professional services provided by Provider:

1. General Terms

1.1. Lesser of Billed Charges

KFHPWA shall compensate Provider, in accordance with Article 4 of this Agreement, with the allowable amount calculated as the lesser of payment per contract compensation terms specified below or eligible billed charges, less any applicable Coinsurance, Copayments, and Deductible amounts.

1.2. RBRVS Calculation

KFHPWA will generally follow CMS RBRVS guidelines, as periodically updated by CMS, for calculation of provider-specific Relative Value Units (RVUs) and related payment methodologies, including but not limited to the correct site of service and geographical adjustments.

2. Medicare Reimbursement

Reimbursement levels for KFHPWA Medicare Members will generally reflect **100%** prevailing Medicare payment levels as they are revised in accordance with the RBRVS and related fee schedules. FQHC and RHC Providers must bill on CMS 1450 (UB04) claim form with corresponding CPT/HCPC codes.

If Provider meets Rural Health Clinic status, the reimbursement levels for KFHPWA Medicare Members will generally reflect Medicare RHC encounter rates. Provider must report status, and provide documentation of such annually, to KFHPWA. Provider will provide KFHPWA with the RHC encounter rates as they receive them from Medicare. Rates will be updated on a prospective basis only. RHC Providers must bill on CMS 1450 (UB04) claim form with corresponding CPT/HCPC codes.

If Provider loses its RHC status during the term of this Agreement, Provider shall notify KFHPWA immediately and agree that the reimbursement will revert to prevailing Medicare payment levels. Claims paid hereunder shall not be subject to an annual reconciliation utilizing Provider's CMS report.

3. Medicaid Reimbursement

Reimbursement levels for KFHPWA Medicaid Members will generally reflect Medicaid payment levels.

4. All Other Lines of Business Reimbursement

(includes all Commercial, non-Medicare and non-Medicaid networks)

- 1.1. The KFHPWA Reimbursement Schedule is generally based on **2024** CMS RBRVS and payment policies and methodology. With the exception of the laboratory services as specified in Section 4.4 of this attachment, reimbursement levels for all other KFHPWA Members shall be based on the KFHPWA Reimbursement Schedule maximum allowable. Drugs furnished incident to physician services shall be reimbursed at **100%** of the KFHPWA Drug Reimbursement Schedule. The KFHPWA Drug Reimbursement Schedule is generally based on the prevailing Medicare Part B Drug Payment Allowance Pricing File updated on a quarterly basis. KFHPWA reserves the right to periodically update the KFHPWA Reimbursement Schedules. The conversion factor is **\$48**.

For Year 2 **February 15, 2026** the conversion factor is **\$48.73**.

For Year 3 **February 15, 2027** the conversion factor is **\$49.48**.

- 4.1. Where CMS does not provide an RBRVS-based RVU, a gap filler methodology will be applied. When no RBRVS-based RVU or gap filler is available, KFHPWA will reimburse at **65%** of eligible billed charges until CMS assigns an RVU or the gap filler service provides a value.
- 4.2. If an update to the RVU year of the KFHPWA Reimbursement Schedule occurs during the term of this contract, the conversion factor may be adjusted to achieve revenue neutrality in aggregate under the updated schedule.
- 4.3. Laboratory Reimbursement
 - A. Laboratory services provided to non-Medicare and non-Medicaid KFHPWA Members shall be reimbursed at the KFHPWA maximum allowable as defined below, contingent upon what is within the scope of Provider's practice.
 - B. Maximum allowable amounts will be based at **100%** of the current Medicare Clinical Lab Fee Schedule.
 - C. If a lab code does not have a fee on the Medicare Clinical Lab Fee Schedule, a gap filler methodology will be applied.
 - D. If a lab code has no fee on the current Medicare Clinical Lab Fee Schedule and no gap filler is available, reimbursement will be based on **65%** of eligible billed charges.

4.4. Radiology Reimbursement

Provider may provide radiology services only if Provider has appropriate certification of radiology equipment, utilizes qualified technicians, and has obtained authorization from KFHPWA when required. If Provider sends x-rays to a radiologist for interpretation, Provider may only bill the technical component.

EXHIBIT FEDERAL PROGRAM COMPLIANCE

A. Medicare Advantage Program

Kaiser Foundation Health Plan of Washington (“Health Plan”) has entered into a Medicare Advantage Organization contract (“MA Contract”) with the Centers for Medicare and Medicaid Services (“CMS”). CMS requires Health Plan to include the provisions of this Section A in its subcontracts. Section A of this Exhibit is incorporated by reference into and made part of the Agreement with respect to Covered Services rendered to Members enrolled in the Medicare Advantage program (“MA Members”) as of the effective date required by law or CMS. In the event of a conflict or inconsistency with any term or condition in the Agreement relating to Covered Services rendered to MA Members, Section A of this Exhibit shall control.

1. **Member Records.** [42 CFR §422.118, §§422.504(a)(13), Medicare Managed Care Manual (“MMCM”) Ch. 11, §§ 100.1 and 100.4] Provider shall (a) abide by all federal and state laws regarding confidentiality and disclosure of medical records, or other health and enrollment information; (b) ensure that medical information is released only in accordance with applicable federal or state law, or pursuant to court orders or subpoenas; (c) maintain medical records and related information in an accurate and timely manner and for ten (10) years after termination or expiration of this Agreement or the date of completion of any audit, whichever is later; and (d) ensure timely access by MA Members to the records and information that pertain to them.
2. **Prompt Payment.** [42 CFR §422.520(b)(1), §422.504(c), MMCM Ch. 6, § 10, MMCM Ch. 11, §100.4] Provider shall be paid for a properly submitted, supported and undisputed claim for Covered Services rendered to MA Members within the time period set forth elsewhere in this Agreement (or if none is specified, then within forty-five (45) calendar days of receipt of such properly submitted, supported and undisputed claim).

3. **Member Hold Harmless.** [42 USC §1395w-22(a)(7), 42 CFR § 422.100(k), §§422.504(g)(1)(i) & (iii), §422.504(i)(3)(i), §422.105(a), §422.100(g)(2), MMCM Ch. 4, §§ 10.2, 50.5, 160 and 170, MMCM Ch. 6, § 10, MMCM Ch. 11, §§ 100.3, 100.4, and 110.3, MMCM Ch. 16-B, § 20.2.4.1] Provider agrees that in no event including, without limitation, nonpayment by or insolvency of Health Plan or breach of this Agreement, shall Provider bill; charge; collect a deposit from; seek compensation, reimbursement, or remuneration from; impose surcharges; or have any recourse against a MA Member or a person acting on behalf of a MA Member for fees that are the legal obligation of Health Plan. This Agreement does not prohibit Provider from collecting Member Cost Share or fees for non-Covered Services to the extent permitted by the applicable health benefit plan; however, Member Cost Share may not be imposed for preventive services that are Covered Services, including influenza and pneumococcal vaccines. If a person who correctly identifies himself as a MA Member seeks Services from Provider without an applicable authorization or referral, Provider may only charge the MA Member for customary in-plan Member Cost Share unless Provider notified the MA Member in advance that the Services would be Covered Services only if further action is taken by the MA Member per applicable health benefit plan rules. If a MA Member is also enrolled in Medicaid and Medicaid is responsible for the Member Cost Share, Provider shall not hold MA Member liable for such Member Cost Share, and Provider shall accept payment pursuant to this Agreement as payment in full or bill Medicaid for such Member Cost Share. Provider may charge an administrative fee for missed appointments only as permitted by law or CMS guidance, including only when Provider charges such administrative fees uniformly to all of its patients. Sections A.3 and A.4 shall be construed in favor of the MA Member as an intended third-party beneficiary, shall survive the termination of the Agreement or the insolvency of Health Plan, and shall supersede any oral or written agreement between Provider and a MA Member.
4. **Continuation of Benefit.** [42 CFR §422.504(g)(2), MMCM Ch. 11, §100.4] In the event of the termination or expiration of this Agreement, insolvency of Health Plan, or other cessation of business, Provider shall continue to provide Covered Services for all MA Members through the period for which premium was paid and, for MA Members who are confined in an inpatient facility on the date of insolvency or termination of Health Plan's MA Contract, through the date of discharge.
5. **Audit and Inspection.** [42 CFR §§422.504 (e)(2) & (i)(2)(i)-(ii) & (iv), MMCM Ch. 11, §100.4, MMCM Ch. 21, § 50.6.1] The Department of Health and Human Services, the U.S. Comptroller General, or their designees have the right to audit, evaluate, collect and inspect any pertinent contracts, books, documents, papers, records (including medical records and patient care documentation), facilities and computer/electronic systems of Provider (including, directly from Provider) involving transactions related to Health Plan's MA Contract during the period of this Agreement and for ten (10) years after termination or expiration of this Agreement or the date of completion of any audit, whichever is later. Provider shall retain such contracts, books, documents, papers, records, and computer/electronic systems for this period.

6. **Accountability and Delegation.** [42 CFR §§422.504(i)(1),(3)(ii),(4)(i)-(iii) & (v) & (5), 42 CFR §422.562(a)(3), MMCM Ch. 6, § 10, MMCM Ch. 11, §§ 100.4 and 110.1 – 110.3, MMCM Ch. 21, §§ 40 and 50.6.6] Health Plan shall only delegate activities or functions to Provider pursuant to a written delegation agreement in compliance with CMS rules, which require, among other things, a covenant of Provider that it will comply with all applicable federal and Medicare laws, regulations, and CMS instructions. To the extent Health Plan delegates any functions for which it is responsible, Health Plan is ultimately responsible to CMS for oversight and compliance and shall monitor performance of the delegated functions on an ongoing basis and to revoke such delegation if Health Plan or CMS determines that performance is unsatisfactory. If Health Plan delegates the selection of providers, Health Plan retains the right to approve, suspend or terminate any such selection.
7. **Exclusion/Sanction and Opt-Out Providers.** [42 USC §§ 1320a-7 & 1320a-7a, 42 CFR §422.204(b)(2)(i), (3) & (4) & (c), §422.220, § 422.222, §422.752(a)(8), §423.120(c)(5)&(6), §498.2, §1001.1901, MMCM Ch. 6, §§ 60.2 & 70, MMCM Ch. 11, § 100.4, MMCM Ch. 21, § 50.6.8, CMS Preclusion List Website: <https://www.cms.gov/Medicare/Provider-Enrollment-and-Certification/Preclusion-List>] Provider represents that (a) it is not excluded, debarred, sanctioned, suspended, precluded or otherwise ineligible from, by or for participation in any federal or state program, including Medicare and Medicaid, (b) with respect to Covered Services provided to MA Members, it does not knowingly employ or contract with an individual or entity so excluded, debarred, sanctioned, suspended, precluded or otherwise ineligible, and (c) no practitioner providing Covered Services to a MA Member has opted out of (*i.e.*, notified CMS that the practitioner has chosen not to participate in) Medicare. If Provider and/or any of its practitioners is an institutional provider or supplier as defined by CMS, Provider and/or any such practitioners providing Covered Services to MA Members shall be enrolled in Medicare. These representations shall be continuing throughout the term of this Agreement, and Provider shall promptly notify Health Plan if any representation can no longer be made. After the expiration of the sixty (60) day period specified in 42 CFR §422.222 (addressing the CMS preclusion list) or as otherwise specified by CMS, Provider shall no longer be eligible for payment from Health Plan for Covered Services rendered to MA Members and shall be prohibited from pursuing payment from such MA Members; and Provider shall hold financial liability for services, items, and drugs that are furnished, ordered, or prescribed to MA Members.
8. **Certification of Data.** [42 CFR §422.504(i)(3), MMCM Ch. 11, §100.4] The chief executive officer of Provider, the chief financial officer, or an individual delegated the authority to sign on behalf of one of these officers, shall certify from time to time, as requested by Health Plan, that the encounter data and other data supplied by Provider (based on its best knowledge, information, and belief) are accurate, complete and truthful.
9. **Termination.** [42 CFR §422.202(d)(4), §422.506, §422.508, §422.510, §422.512, MMCM Ch. 4, §§ 110.1.2.2-110.1.2.4, MMCM Ch. 6, §60.4, MMCM Ch. 11, §§ 60, 70, 80, 90 & 100.4] If this Agreement may be terminated without cause, the minimum period of notice shall be at least ninety (90) days but shall be greater if provided in this Agreement. If the MA Contract between Health Plan and CMS is terminated or not renewed, this Agreement will be terminated as to MA Members, except to the extent Health Plan enters into a different form of contract with CMS, in which case Provider agrees to cooperate with Health Plan in meeting its requirements under the new contract with CMS until such time as this Agreement may be amended.

10. **Access to Books and Records.** [42 USC §1395x(v)(1)(I), 42 CFR §420.302(b)] If this Agreement is determined to be subject to the provisions of 42 USC §1395x(v)(1)(I), Provider agrees to permit representatives of the Secretary of the U.S. Department of Health and Human Services and the U.S. Comptroller General, and their duly authorized representatives, to have access to this Agreement and to the books, documents, and records of Provider, until the expiration of four (4) years after the services are furnished under this Agreement, and as necessary to verify the costs of this Agreement in accordance with criteria and procedures contained in applicable federal law.
11. **Advance Directives.** [42 CFR §§422.128(b)(1)(ii)(E)&(F), MMCM Ch. 4, § 180, MMCM Ch. 11, § 100.4] The MA Member's medical record shall reflect, in a prominent part, whether or not the MA Member has executed an advance directive. Provider may not condition the provision of care or otherwise discriminate against a MA Member based on whether or not the MA Member has executed an advance directive. Subject to applicable state law, nothing in this section shall require Provider to implement an advance directive to which Provider conscientiously objects.
12. **Compliance.** [42 CFR §§422.504(a)(8), (h) & (i)(3)(iii), §422.310(b), §422.516(a), §422.503(b)(4)(v)-(vi), MMCM Ch. 4, § 10.5.3, MMCM Ch. 6 § 10, MMCM Ch. 11, §§ 20.3, 100.4, 110.1-2 and 120, MMCM Ch. 21, §§ 30, 40, 50.1.3, 50.3, 50.4, 50.6 (in particular, 50.6.6 & 50.6.11) & 50.7.2] Provider shall comply and shall require any subcontractors providing services to MA Members, to comply with all applicable Medicare laws and regulations (including without limitation those designed to prevent or ameliorate fraud, waste and abuse), state and federal laws (including criminal laws, the False Claims Act, Anti-Kickback statute, Health Insurance Portability and Accountability Act or HIPAA, Civil Rights Act of 1964, Age Discrimination Act of 1975, Rehabilitation Act of 1973, Americans with Disabilities Act, Genetic Information Nondiscrimination Act of 2008), with CMS guidance and instructions, with Health Plan's policies and procedures, with applicable elements of Health Plan's compliance program (including, without limitation, reporting of compliance issues, cooperation with Health Plan's routine monitoring and auditing of providers, and annual training and education, e.g., related to fraud, waste and abuse), and with applicable contractual obligations under the Health Plan's MA Contract, as amended from time to time. Failure to comply with Health Plan's compliance program may result in a corrective action or other appropriate action under the Agreement. In the event of changes to the governing laws, regulations, or CMS requirements applicable to the Medicare Advantage program, this Exhibit shall be amended to the extent required by any such later required changes. Provider shall cooperate, assist, and provide records, data, and information, as requested by Health Plan, for Health Plan's compliance with Medicare requirements.

- A. **Offshore Subcontracting.** [HPMS Memorandum, Sponsor Activities Performed Outside of the United States (Offshore Subcontracting) (July 23, 2007), HPMS Memorandum, Sponsor Activities Performed Outside of the United States (Offshore Subcontracting) Questions & Answers (Sept. 20, 2007), HPMS Memorandum, Offshore Subcontractor Data Module in HPMS (Aug 26, 2008)] Provider shall provide information to Health Plan about disclosure of MA Members' Protected Health Information ("PHI," as defined by HIPAA) to entities outside the United States so that Health Plan may complete CMS's required Offshore Subcontractor Information and Attestation form; and if Provider discloses MA Members' PHI to entities outside the United States, Provider shall inform Health Plan of the fact of such disclosures within fifteen (15) days of contract execution or amendment, shall implement reasonable security policies and procedures auditable by Health Plan to protect such PHI, and shall report actual or suspected security breaches to Health Plan.
 - B. **Federal Funds.** [MMCM Ch. 11, § 100.4] Provider acknowledges that funds received from Health Plan are in whole or in part derived from federal funds.
 - C. **Grievances and Appeals.** [42 CFR §422.562(a), MMCM Ch. 11, § 100.4] Provider shall also cooperate with Health Plan's grievance and appeals procedures for MA Members.
 - D. **Communications and Marketing.** [42 CFR §§422.2260-2276 (particularly 42 CFR §422.2266), *see generally* Medicare Communications and Marketing Guidelines ("MCMG") (particularly "Activities with Healthcare Providers or in the Healthcare Setting")] If Provider engages in any communications or marketing activities related to MA Members or the Medicare Advantage program (including distribution of any materials related to the Medicare Advantage program), Provider shall comply with all applicable Medicare Advantage communications and marketing rules, including remaining neutral if and when assisting Members with Medicare Advantage and/or Prescription Drug Plan enrollment decisions.
 - E. **Provider Information.** [45 CFR § 162.410] Provider shall maintain complete and accurate information in the CMS National Plan and Provider Enumeration System ("NPES").
13. **Credentialing.** [42 CFR §422.20 4(a) & (b)(1)-(2), §422.112(a)(5), §422.504(i)(4), MMCM Ch. 6, §§ 60.3 & 70] Provider agrees to cooperate with Health Plan's credentialing process for providers rendering Covered Services to MA Members (including recredentialing at least every 3 years). Provider agrees that Health Plan will review the credentials of Provider and (as applicable) its health care providers or will allow Health Plan to review, approve and audit Provider's credentialing process.

14. **Access to Services.** [42 CFR §§422.100(g)(1), §§422.112(a)(1),(3),(6),(7),(8) & (9), §422.110(a), §422.113(b), §422.206(a)(2), §422.504(a)(2), MMCM Ch. 4, §§ 20, 10.5.2, 50.2 & 110, MMCM Ch. 6, § 40, MMCM Ch. 11, §100.4] Covered Services shall be available and accessible in a timely manner, during hours of operation convenient to the population served, and in a manner that does not discriminate against MA Members. Provider shall not discriminate against MA Members on the basis of health status (including medical condition, claims experience, receipt of health care, medical history, genetic information, evidence of insurability, conditions arising out of acts of domestic violence and disability), race, ethnicity, national origin, religion, sex, age, mental or physical disability, sexual orientation, or source of payment. Covered Services and information about treatment options shall be provided to MA Members in a culturally competent manner, including the option of no treatment, and with appropriate assistance for MA Members with limited communication skills and disabilities. Provider shall allow direct access (a) for all MA Members to influenza vaccines and (b) for women MA Members to screening mammography and women's health specialists for women's routine and preventive health care services. Health Plan shall assume financial responsibility for emergency and urgently needed services to MA Members in accordance with applicable law and health benefit plan rules, regardless of whether there is a prior authorization for the services.
15. **Quality Assurance.** [42 CFR §422.112(b)(5), §422.152(a), §422.504(a)(3)(iii) & (5), *see generally* MMCM Ch. 5, MMCM Ch. 11, §§ 100.1 & 100.4] Provider shall participate in and cooperate with Health Plan's quality assurance and improvement programs, including cooperating with any independent or external review organization retained by Health Plan as part of its quality assurance and improvement programs. Provider shall render Covered Services in a manner consistent with professionally recognized standards of care. Provider shall inform MA Members of specific health care needs that require follow-up and shall provide, as appropriate, training in self-care and other measures for MA Members to promote their own health.
16. **Subcontractors.** [42 CFR §422.504(i)(3)(iii), MMCM Ch. 11, § 110.1] If Provider provides Covered Services to MA Members through a subcontractor, Provider shall require such subcontractor to provide Covered Services to MA Members consistent with Health Plan's contractual obligations.
17. **Encounter Reporting.** [42 CFR §422.310] In the event Provider does not submit standard claims for payment, Provider shall provide the information necessary for Health Plan to report to CMS all encounters for MA Members on a standard CMS 1500 or UB-04 form.
18. **Hospital Provisions.** [42 CFR §422.620, Parts C & D Enrollee Grievances, Organization/Coverage Determinations, and Appeals Guidance, § 100] If Provider is a hospital, Provider shall provide MA Members with advance notice of hospital discharge appeal rights and cooperate with Health Plan and/or the applicable Quality Improvement Organization regarding appeals or grievances related to the discharge.
19. **Physician Provisions.** [42 CFR §§422.208-210, §422.202(d)(1), MMCM Ch. 11, §100.4, MMCM Ch.6 §§ 60.4 and 80] Where Provider is a physician or physician group: (a) if the compensation arrangement places physician(s) at "substantial financial risk" as defined under the Physician Incentive Plan rules, the parties shall comply with such rules; and (b) if this Agreement is suspended or terminated, physician(s) shall be given written notice of the reasons for suspension or termination and, if applicable, the right to appeal.

B. Federal Employees Health Benefits Program and Postal Service Health Benefits Program

Health Plan has entered into contracts with the U.S. Office of Personnel Management (“OPM”) (“FEHBP Contract” and “PSHBP Contract”) to provide or arrange health care services for persons enrolled in the Federal Employees Health Benefits Program (“FEHBP”) and the Postal Service Health Benefits Program (“PSHBP”). OPM requires Health Plan to include the provisions of this Section B in any subcontracts. Section B of this Exhibit is incorporated by reference into and made part of the Agreement with respect to Covered Services rendered to Members enrolled in FEHBP (“FEHBP Members”) and PSHBP (“PSHBP Members”) as of the effective date required by law or OPM. In the event of a conflict or inconsistency with any term or condition in the Agreement relating to Covered Services rendered to FEHBP Members or PSHBP Members, Section B of this Exhibit shall control.

1. **Service Obligations.** [FEHBP Contract/PSHBP Contract §§1.9, 1.11, 1.20, and 1.26] Provider and its health care providers shall cooperate with Health Plan quality standards, implementation of patient safety improvement programs and disaster recovery plans and assist Health Plan with collection of data for quality assurance records.
2. **Hold Harmless.** [FEHBP Contract/PSHBP Contract §2.9] In the event of (a) insolvency of Health Plan or of Provider, or (b) Health Plan’s or Provider’s inability to pay expenses for any reason, Provider shall not look to FEHBP Members/PSHBP Members for payment and shall prohibit health care providers from looking to FEHBP Members/PSHBP Members for payment.
3. **Billing and Payment.** [FEHBP Contract/PSHBP Contract §§2.3(g), 2.6(b), and 2.11] Provider shall cooperate with Health Plan in the performance of its obligations under the FEHBP Contract/PSHBP Contract to administer and coordinate benefits, pay claims, and recoup erroneous payments within the time frames permitted or required by OPM. Provider shall submit claims on the appropriate CMS 1500 form or UB-04 form and shall make all reasonable efforts to submit claims electronically.
4. **Termination of FEHBP Contract/PSHBP Contract.** [FEHBP Contract/PSHBP Contract §5.49] If the FEHBP Contract and/or PSHBP Contract is terminated by OPM, the Agreement and all subcontracts shall be terminated with respect to FEHBP Members and/or PSHBP Members, as applicable, addressed by such contract, and the parties shall assign to the government, as directed by OPM, all right, title, and interest of Health Plan under the Agreement and subcontracts terminated.
5. **Continuation of Care.** [FEHBP Contract/PSHBP Contract §1.24] In the event Health Plan terminates its FEHBP Contract and/or PSHBP Contract with OPM or terminates this Agreement with Provider other than for cause, Provider and Health Plan agree that specialized care (if any is provided under this Agreement) shall continue to be rendered and paid under the terms of this Agreement for those FEHBP Members and/or PSHBP Members, as applicable, who are undergoing treatment for a chronic or disabling condition or who are in the second or third trimester of pregnancy for up to ninety (90) days, or through their postpartum period, whichever is later. Provider shall also promptly transfer all medical records to the designated new provider during or upon completion of the transition period, as authorized by the FEHBP Member and/or PSHBP Member, as applicable, and shall give all necessary information to Health Plan for quality assurance purposes.

6. **Confidentiality.** [FEHBP Contract/PSHBP Contract §1.6(b)] Provider shall hold confidential all medical records of FEHBP Members/PSHBP Members, and information relating thereto, except (a) as may be reasonably necessary for administration of the FEHBP Contract/PSHBP Contract, (b) as authorized by the FEHBP Member/PSHBP Member or his or her guardian, (c) as disclosure is necessary to permit government officials having authority to investigate and prosecute alleged civil or criminal actions, (d) as necessary to audit the FEHBP Contract/PSHBP Contract, (e) as necessary to carry out the coordination of benefit provisions of the FEHBP Contract/PSHBP Contract, or (f) for bona fide medical research or educational purposes (only if aggregated).
7. **Maintenance and Audit of Records.** [FEHBP Contract/PSHBP Contract §§3.4, 5.7 and 48 CFR §§2.101, 52.215-2] OPM and other government officials have the right to inspect and evaluate the work performed or being performed under the FEHBP Contract/PSHBP Contract, records involving work or transactions related to the FEHBP Contract/PSHBP Contract, and the premises where the work is being performed, at all reasonable times and in a manner that will not unduly delay the work. If government officials or their authorized representatives request access, inspection or evaluation of such Provider records or premises, Provider shall cooperate by providing access to records and facilities until six years after final payment or settlement under the FEHBP Contract/PSHBP Contract.
8. **Notice of Significant Events.** [FEHBP Contract/PSHBP Contract §1.10 and 48 CFR §1652.222-70] Provider agrees to notify Health Plan of any Significant Event within seven (7) working days after the Provider becomes aware of it. A “Significant Event” is any occurrence or anticipated occurrence that might reasonably be expected to have a material effect upon Provider’s ability to meet its obligations under the Agreement.
9. **Compliance.** [FEHBP Contract/PSHBP Contract §§ 1.20, 1.38, 2.7, 5.5, 5.18, 5.19, 5.22, 5.23, 5.45, 5.47, 5.55, 5.56, 5.57, 5.59, 5.61, 5.64, 5.65, 5.69, 5.70, 5.71, 5.72, 5.73, 5.74, 5.75]

Provider and Health Plan shall comply with the Health Care Consumer Bill of Rights (at <https://www.opm.gov/healthcare-insurance/healthcare/consumer-protections/#url=Bill-of-Rights>), as amended from time to time, which addresses FEHBP Members’/PSHBP Members’ ability to participate fully in treatment decisions, respecting FEHBP Members’/PSHBP Members’ rights (including nondiscrimination), and protecting FEHBP Members’/PSHBP Members’ privacy. [FEHBP Contract/PSHBP Contract §1.20] Provider shall not employ or contract with any providers that provide Covered Services to FEHBP Members/PSHBP Members and have been debarred, suspended, or proposed for debarment by the federal government during the term of the Agreement. [FEHBP Contract/PSHBP Contract §5.47 and 48 CFR §52.209-6 (NOV 2021)] Health Plan shall not be liable for payment to Provider for services rendered by a provider debarred, excluded, or suspended from participation in any federal program. [FEHBP Contract/PSHBP Contract §2.7] In addition, as requested, Provider shall cooperate with, assist, and provide information to Health Plan as needed for Health Plan’s compliance with all FEHBP Contract/PSHBP Contract requirements.

- A. Health Plan is subject to various federal laws, executive orders and regulations regarding equal opportunity and affirmative action. This subsection 9A constitutes notice that Provider may be subject to the following Federal Acquisition Regulations (each a “FAR”) at 48 CFR Part 52 and the Office of Federal Contract Compliance Regulations at 41 CFR Part 60, which are incorporated herein by reference: (i) FEHBP Contract/PSHBP Contract §5.19 corresponding to FAR 52.222-26 – Equal Opportunity (SEP 2016) and 41 CFR 60.1.4(a); (ii) FEHBP Contract/PSHBP Contract §5.22 corresponding to FAR 52.222-35 – Equal Opportunity for Veterans (JUN 2020) and 41 CFR 60-300.5(a); (iii) FEHBP Contract/PSHBP Contract §5.23 corresponding to FAR 52.222-36 – Equal Opportunity for Workers with Disabilities (JUN 2020) and 41 CFR 60-741.5(a); (iv) FEHBP Contract/PSHBP Contract §5.55 corresponding to FAR 52.222-37 – Employment Reports on Veterans (JUN 2020); and (v) FEHBP Contract/PSHBP Contract §5.59 corresponding to FAR 52.222-21 – Prohibition of Segregated Facilities (APR 2015) and 41 CFR 60-1.8, which provide (and are required to be stated in bold print): **“This contractor [Health Plan] and subcontractor [Provider, if covered] shall abide by the requirements of 41 CFR 60-300.5(a) and 60-741.5(a). These regulations prohibit discrimination against qualified individuals on the basis of protected veteran status or disability and require affirmative action by covered prime contractors and subcontractors to employ and advance in employment qualified protected veterans and qualified individuals with disabilities.”** In addition, per FEHBP Contract/PSHBP Contract §§5.19 and 5.61, Executive Order 11246 regarding nondiscrimination in employment decisions, as amended by Executive Order 13665 regarding non-retaliation for disclosure of compensation information, and Executive Order 13496 (codified at 29 CFR Part 471, Appendix A to Subpart A) concerning the obligations of federal contractors and subcontractors to provide notice to employees about their rights under Federal labor laws shall be incorporated herein by reference. As part of Health Plan’s efforts to comply with these requirements, Health Plan has developed and implemented equal employment opportunity and affirmative action policies and programs designed to ensure that all qualified applicants and employees are treated without regard to such factors as race, color, religion, sex, sexual orientation, gender identify, national origin, disability, veteran status, or any other reason prohibited by law. Health Plan requests that, as one of Health Plan’s subcontractors, Provider take appropriate action, as necessary, to support Health Plan’s commitment to these requirements, as required by 41 CFR 60-300.44(f)(1)(ii) and 60-741.44(f)(1)(ii).

In addition, this subsection 9A constitutes notice that Provider may be subject to additional FARs, which are incorporated herein by reference: (a) FEHBP Contract/PSHBP Contract §5.5 corresponding to FAR 52.203-7 – Anti-Kickback Procedures (JUN 2020); (b) FEHBP Contract/PSHBP Contract §5.18 corresponding to FAR 52.222-4 – Contract Work Hours and Safety Standards Act – Overtime Compensation (MAY 2018); (c) FEHBP Contract/PSHBP Contract §5.45 corresponding to FAR 52.203-12 – Limitation on Payments to Influence Certain Federal Transactions (JUN 2020); (d) FEHBP Contract/PSHBP Contract §5.56 corresponding to FAR 52.227-1 – Authorization and Consent (JUN 2020); (e) FEHBP Contract/PSHBP Contract §5.57 corresponding to FAR 52.227-2 – Notice and Assistance Regarding Patent and Copyright Infringement (JUN 2020); (f) FEHBP Contract/PSHBP Contract §5.64 corresponding to FAR 52.203-13 – Contractor Code of Business Ethics and Conduct (NOV 2021); (g) FEHBP Contract/PSHBP Contract §5.69 corresponding to FAR 52.223-18 – Encouraging Contractor Policies to Ban Text Messaging While Driving (JUN 2020); (h) FEHBP Contract/PSHBP Contract §5.70 corresponding to FAR 52.203-17 – Contractor Employee Whistleblower Rights and Requirement to Inform Employees of Whistleblower Rights (JUN 2020); (i) FEHBP Contract/PSHBP Contract §5.71 corresponding to FAR 52.222-50 – Combating Trafficking in Persons (NOV 2021); (j) FEHBP Contract/PSHBP Contract §5.72 corresponding to FAR 52.204-21 – Basic Safeguarding of Covered Contractor Information Systems (NOV 2021); (k) FEHBP Contract/PSHBP Contract §5.73 corresponding to FAR 52.203-19 – Prohibition on Requiring Certain Internal Confidentiality Agreements or Statements (JAN 2017); (l) FEHBP Contract/PSHBP Contract §5.74 corresponding to FAR 52.204-23 – Prohibition on Contracting for Hardware, Software, and Services Developed or Provided by Kaspersky Lab and Other Covered Entities (NOV 2021); (m) FEHBP Contract/PSHBP Contract §5.75 corresponding to FAR 52.204-25 – Prohibition on Contracting for Certain Telecommunications and Video Surveillance Services or Equipment (NOV 2021); (n) FEHBP Contract/PSHBP Contract §5.77 corresponding to FAR 52.204-27 – Prohibition on a ByteDance Covered Application (JUN 2023); and (o) FEHBP Contract/PSHBP Contract §5.65 corresponding to FAR 52.222-54 – Employment Eligibility Verification (MAY 2022).

If Provider is not otherwise subject to compliance with the laws and executive orders specified in this subsection 9A, the inclusion of this subsection 9A shall not be deemed to impose such requirements upon Provider.

10. **Health Information Technology.** [FEHBP Contract/PSHBP Contract §1.27] As Provider implements, acquires, or upgrades health information technology systems, it shall use reasonable efforts to utilize, where available, certified health information technology systems and products that meet interoperability standards recognized by the Secretary of Health and Human Services (“Interoperability Standards”), have already been pilot-tested in a variety of live settings, and demonstrate meaningful use of health information technology in accordance with the HITECH ACT. Provider shall also encourage its subcontracted providers to comply with applicable Interoperability Standards.
11. **Licensure and Other Credentials.** [FEHBP Contract/PSHBP Contract §1.9(f)] Provider shall require that all physicians providing Covered Services to FEHBP Members/PSHBP Members comply with Health Plan’s credentialing requirements.

12. **Surprise Billing and Transparency.** [FEHBP Contract/PSHBP Contract §2.18] Health Plan shall extend the provisions of sections 2799B-6, 2799B-8, and 2799B-9 of the Public Health Service Act to FEHBP Members/PSHBP Members in the same manner as such provisions apply to an enrollee in a group health plan or group or individual health insurance coverage offered by a health insurance issuer.

C. Affordable Care Act and Public Health Service Act

1. **Exchanges.** [45 CFR § 156.340] To the extent Provider provides Services to, or performs other responsibilities specified in this Agreement with respect to, Members enrolled in fully-insured Qualified Health Plans (“QHPs”) offered by Health Plan through the Federally-Facilitated Exchange (“FFE”), the State-Based Exchanges using the technology of the Federally-Facilitated Exchange (“SBE-FPs”) and/or the State-Based Exchanges (“SBEs”) pursuant to the Patient Protection and Affordable Care Act of 2010, Pub. L. No. 111-148, as amended, and implementing regulations including 45 CFR Section 156.340, other regulations, and/or subregulatory guidance issued by CMS (collectively referred to as the “ACA”, and such Members referred to as “Exchange Members”, in this Section C.1), Provider agrees the following provisions shall apply. This Section C.1 is incorporated into and made part of the Agreement with respect to Covered Services rendered to Exchange Members as of the effective date required by law.
 - A. **Delegated Entity.** In the course of performing the duties and obligations set forth in the Agreement (collectively referred to as “Duties”), Provider acts as a “delegated entity” and if Provider subcontracts with other entities as may be allowed by the Agreement, such other entities constitute “downstream entities,” as such terms are defined in 45 C.F.R. § 156.20, to assist in performing its Duties.
 - B. **Compliance with Laws.** Provider shall comply with all applicable laws and regulations, including but not limited to the provisions of 45 C.F.R. Parts 155 and 156, to the extent relevant, in performing its Duties including but not limited to the privacy and security standards for Non-Exchange Entities (as that term is defined in Part 155) and, when handling the personally identifiable information (PII) of Exchange Members and applicants, the terms and conditions set forth in Health Plan’s Enhanced Direct Enrollment Agreement for the FFE and SBE-FPs.
 - C. **Access to Books and Records.** Provider shall grant access to its books, contracts, computers, or other electronic systems (including medical records and documentation), relating to Provider’s compliance with applicable provisions under 45 C.F.R. Parts 155 and 156 in connection with its Duties, to Health Plan, the FFE, SBE-FP or SBE, as applicable, and the U.S. Department of Health and Human Services (“HHS”) and its Office of Inspector General (or their designees), for the duration of the period in which the Agreement is effective, and for a minimum of ten (10) years from the date the Agreement terminates.

D. **Downstream Subcontracts.** Provider shall include in its contracts with any downstream entities, and require such downstream entities to include in their contracts with other downstream entities, language that is the same or substantially similar to that contained in this Section C.1 and which expressly requires each downstream entity to: (i) comply with all applicable laws and regulations, including but not limited to the provisions of 45 C.F.R. Parts 155 and 156 and Health Plan's Enhanced Direct Enrollment Agreement, to the extent relevant, in performing or assisting in the performance of Duties; and (ii) grant access to its books, contracts, computers, or other electronic systems (including medical records and documentation), relating to such downstream entity's compliance with applicable provisions under 45 C.F.R. Parts 155 and 156, in connection with the Duties, to Health Plan and HHS and its Office of Inspector General (or their designees), for the duration of the period in which the Agreement or subcontract (as applicable) is effective, and for a minimum of ten (10) years from the date the Agreement or subcontract (as applicable) terminates.

E. **Termination and Right to Revoke.** In the event that HHS or KP determines that Provider, or any downstream entity with whom/which Provider contracts as described in this Section C, has not satisfactorily performed the Duties, Health Plan shall have the right to revoke such Duties and immediately terminate the Agreement and/or subcontract (as applicable) for cause with respect to the Health Plan's QHPs offered through the FFE, SBE-FP and/or SBE and the Exchange Members.

2. **Prohibition on Gag Clauses.** This Section C.2 is incorporated by reference into and made part of the Agreement as of the effective date required by law.

A. **Prohibition on Gag Clauses on Price and Quality Information in Provider Agreements for Group Health Plans and Group Health Insurance Coverage.** [Internal Revenue Code Section 9824; Employee Retirement Income Security Act ("ERISA") Section 724; and Public Health Service Act Section 2799A-9(a)(1)] Notwithstanding anything to the contrary in the Agreement, with respect to group health plans and group health insurance or group health plan coverage, Health Plan in its capacity either as an issuer or service provider offering access to a network of providers shall not be directly or indirectly restricted from:

- (i) providing provider-specific cost or quality of care information or data, through a consumer engagement tool or any other means, to referring providers, the plan sponsor, participants, beneficiaries, or enrollees, or individuals eligible to become participants, beneficiaries, or enrollees of the plan or coverage;
- (ii) electronically accessing de-identified claims and encounter information or data for each participant, beneficiary, or enrollee in the plan or coverage upon request and consistent with the privacy regulations promulgated pursuant to section 264(c) of HIPAA, the Genetic Information Nondiscrimination Act of 2008 ("GINA"), and the Americans with Disabilities Act of 1990 ("ADA"), including, on a per claim basis
 - (a) financial information, such as the allowed amount, or any other claim-related financial obligations included in the Agreement;
 - (b) provider information, including name and clinical designation;
 - (c) service codes; or
 - (d) any other data element included in claim or encounter transactions; or

- (iii) sharing information or data described in subsections C.2.A.i and C.2.A.ii, or directing such information be shared, with a business associate, as defined in 45 CFR 160.103, consistent with applicable privacy regulations promulgated pursuant to section 264(c) of HIPAA, GINA, and the ADA.

B. Prohibition on Gag Clauses on Price and Quality Information in Provider Agreements for Individual Health Insurance Coverage. [Public Health Service Act Section 2799A-9(a)(2)] Notwithstanding anything to the contrary in the Agreement, with respect to individual health plan coverage, Health Plan in its capacity either as an issuer or service provider offering access to a network of providers shall not be directly or indirectly restricted from:

- (i) providing provider-specific price or quality of care information, through a consumer engagement tool or any other means, to referring providers, enrollees, or individuals eligible to become enrollees of the plan or coverage; or
- (ii) sharing, for plan design, plan administration, and plan, financial, legal, and quality improvement activities, data described in subsection C.2.B.i with a business associate, consistent with the privacy regulations promulgated pursuant to section 264(c) of HIPAA, GINA, and the ADA.

D. Miscellaneous

1. **Hospice Provisions.** [42 CFR §418.114(d)(1) and (2)] If Provider delivers Covered Services to patients receiving hospice care, Provider shall ensure that all its employees who have direct contact with such patients or access to such patients' records have had a criminal background check in accordance with applicable state law requirements. In the absence of state law requirements, Provider will obtain criminal background checks within three (3) months of the date of employment for all states that the individual has lived or worked in the previous three (3) years.

EXHIBIT **NATIONAL COMMITTEE FOR QUALITY ASSURANCE** **("NCQA") REQUIRED PROVISIONS**

This Exhibit is incorporated by reference into and made part of the Agreement with respect to Services rendered to Members of Kaiser Foundation Health Plan of Washington ("KFHPWA"), as required by NCQA. The obligations under this Exhibit are the responsibility of KFHPWA.

1. Quality Improvement. Provider shall participate in KFHPWA's Quality Improvement ("QI") program, including cooperating with QI activities, providing applicable performance data, and tracking and regular reporting on mutually agreed upon quality indicators, all in accord with KFHPWA's expectations and NCQA standards.
2. Communications with Members. Provider's physicians and other personnel licensed or certified to provide services to Members hereunder may freely communicate with a Member or a Member's authorized representatives about the Member's treatment options, without regard to benefit coverage limitations, while maintaining confidentiality consistent with the confidentiality provisions set forth in this Agreement.
3. Utilization Management Decisions. Utilization management decision-making is based on appropriateness of care and service and existence of coverage. Individuals responsible for utilization management decision-making do not receive financial incentives that specifically reward them for issuing denials of coverage or service, or that encourage decisions that result in underutilization.
4. Credentialing. Provider shall comply and shall cause its physicians and other personnel to comply, with KFHPWA's credentialing requirements. KFHPWA retains the right, based on quality issues, to approve new practitioners, providers, and sites and to terminate or suspend the right of individual practitioners, providers, or sites to treat Members.
5. Confidentiality. Provider shall maintain the confidentiality of Member information and records, and all other protected health information, in compliance with the confidentiality provisions set forth in this Agreement and Policies regarding protected health information. Data shared with employers, whether self-insured or insured, shall not implicitly or explicitly identify a Member without the written consent of the Member, except as permitted by law.
6. Site Visits and Medical Records Reviews. Provider shall permit and cooperate with, at reasonable times with reasonable notice, initial and follow-up inspection of its site(s) by representatives of KFHPWA, NCQA and other accrediting or licensing organizations on a biannual basis or more frequently as requested by KFHPWA. Provider shall cause its physicians and other personnel to permit and cooperate with such inspections and medical record reviews.
7. Subcontracts. Provider shall require all provisions of this Exhibit to be included in any contract or agreement between Provider and any subcontractor providing services for Members.

EXHIBIT WASHINGTON STATE COMPLIANCE

This Exhibit is incorporated by reference into and made part of the Agreement with respect to Services rendered to Members enrolled under Washington group or individual coverage issued by Kaiser Foundation Health Plan of Washington (“KFHPWA”). In the event of a conflict or inconsistency with any term or condition in the Agreement relating to Services rendered to Members enrolled under Washington group or individual coverage issued by KFHPWA, this Exhibit shall control.

Network shall itself, or shall cause KFHPWA to, satisfy the obligations of KFHPWA under this Exhibit. With respect to Members of KFHPWA, KFHPWA is the entity ultimately responsible for the obligations under this Exhibit.

With respect to Services rendered to Members enrolled under Washington group or individual coverage issued by KFHPWA, this Agreement shall be governed by the laws of the state of Washington, without application of the conflict of law’s provisions of the state of Washington.

1. **Administrative Policies.** [WAC 284-170-421(5)&(6)] In order to comply with Washington law, the following is added to the provision regarding Policies set forth in Article 2 of the Agreement:

KFHPWA shall notify Provider of Provider’s responsibilities with respect to KFHPWA's applicable administrative policies and programs including, but not limited to, payment terms, utilization review, quality assessment and improvement programs, credentialing, prior authorizations and method used to accept prior authorization requests, grievance, appeal and adverse benefit determination procedures, data reporting requirements, pharmacy benefit substitution processes, confidentiality requirements and any applicable federal or state requirements. Documents, procedures, and other administrative policies and programs referenced in the Agreement shall be available for review by Provider prior to the Effective Date of this Agreement.

2. **Audit Guidelines.** [WAC 284-170-460] In order to comply with Washington law, the following is added to the provision regarding access to records set forth in Article 7 of the Agreement:

KFHPWA shall not have access to health information and other similar records of Provider unrelated to Members. This provision shall not limit KFHPWA’s right to ask for and receive information relating to the ability of the Provider to deliver health care services that meet the accepted standards of medical care prevalent in the community. Any access to patient records of Provider by KFHPWA shall be limited to only that necessary to perform the audit. Provider shall have claims audit rights that are reciprocal to KFHPWA’s claims audit rights in the Agreement.

3. **Clean Claims.** [WAC 284-170-431] In order to comply with Washington law, the following is added to the provision regarding claims set forth in Article 4 of the Agreement:

Provider shall be paid as soon as practical, subject to the following minimum standards: (a) 95% of the monthly volume of clean claims shall be paid within thirty (30) days of receipt, and (b) 95% of the monthly volume of all claims shall be paid or denied within sixty (60) days of receipt, except as agreed to in writing by the parties on a claim-by-claim basis. The receipt date of a claim is the date KFHPWA, or its designee receives either written or electronic notice of the claim. KFHPWA or its designee shall establish a reasonable method for confirming receipt of claims and responding to provider inquiries about claims. If clean claims are not paid within the standard established under the Agreement, KFHPWA shall pay interest on undenied claims and unpaid clean claims more than sixty-one (61) days old until the standard established under the Agreement is met. Interest shall be assessed at the rate of one percent per month and shall be calculated monthly as simple interest prorated for any portion of a month. KFHPWA shall add the interest payable to the amount of the unpaid claim without the necessity of the Provider submitting an additional claim. Any interest paid under this Section shall not be applied by KFHPWA to a Member's deductible, copayment, coinsurance, or any similar obligation of the Member. When KFHPWA or its designee issues payment in the Provider's and the Member's names, KFHPWA shall make claim checks payable in the name of the Provider first and the Member second. For purposes of this Section, "clean claim" means a claim that has no defect or impropriety, including any lack of any required substantiating documentation, or particular circumstances requiring special treatment that prevents timely payments from being made on the claim under this Section. Denial of a claim shall be communicated to the Provider and shall include the specific reason why the claim was denied. If the denial is based upon medical necessity or similar grounds, then KFHPWA or its designee upon request of the Provider shall also promptly disclose the supporting basis for the decision. KFHPWA shall be responsible for ensuring that any person acting on behalf of or at the direction of KFHPWA complies with these billing and claim payment standards. These standards do not apply to claims about which there is substantial evidence of fraud or misrepresentation by Provider or the Member, or instances where KFHPWA has not been granted reasonable access to information under the Provider's control. Provider, KFHPWA, and KFHPWA's designees are not required to comply with these contract provisions if the failure to comply is occasioned by any act of God, bankruptcy, act of a governmental authority responding to an act of God or other emergency, or the result of a strike, lockout, or other labor dispute.

4. **Provider Credentialing Reimbursement Requirements.** [RCW 48.43.757] In order to comply with Washington law, the following is added to the provision regarding compensation set forth in Article 4 of the Agreement:

Notwithstanding any provision to the contrary, compensation to Provider shall follow the requirements of RCW 48.43.757 regarding provider credentialing applications.

5. **Contracting Outside Plan.** [RCW 48.43.085; RCW 48.43.016] In order to comply with Washington law, the following is added to the provision regarding member hold harmless set forth in Article 4 of the Agreement:

Notwithstanding any other provision of law, KFHPWA may not directly or indirectly prohibit Members from freely contracting at any time to obtain any health care services outside their health care plan on any terms or conditions the Members choose. Nothing in this Section shall be construed to bind KFHPWA for any services delivered outside the health plan. KFHPWA may not require Provider to provide a discount from usual and customary rates for health care services that are not Covered Services, and any such provision in the Agreement shall not apply to Members addressed by this Exhibit.

6. **Contract Termination.** [WAC 284-170-421(9)&(10)] In order to comply with Washington law, the following is added to Article 5 of the Agreement (Term and Termination):

The minimum period of notice of termination without cause by either party shall be at least sixty (60) days but shall be greater if so provided elsewhere in this Agreement. At least thirty (30) days prior to the effective date of termination, or immediately for a termination for cause that results in less than thirty (30) days' notice, KFHPWA shall make a good faith effort to ensure that written notice of the termination is provided to all Members who are patients seen on a regular basis by a specialist provider, or who have a standing referral to Provider, or in the case of a primary care provider, to all Members who are patients of Provider, irrespective of whether the termination was for cause or without cause.

7. **Continuity of Care following Contract Termination.** [RCW 48.43.515(7)] In order to comply with Washington law, the following is added to the provision regarding effects of termination set forth in Article 5 of the Agreement:

KFHPWA must cover services of a primary care provider whose contract with the plan or whose contract with a subcontractor is being terminated without cause under the terms of that contract for at least sixty (60) days following notice of termination to Members or, in group coverage arrangements involving periods of open enrollment, only until the end of the next open enrollment period.

8. **Member Coverage; Non-discrimination.** [RCW 48.46.090; RCW 48.46.110(2); WAC 284-170-421(11)] In order to comply with Washington law, the following is added to the provision regarding non-discrimination set forth in Article 2 of the Agreement:

Provider shall furnish Covered Services to Members without regard to the Member's enrollment in the plan as a private purchaser of the plan or as a participant in publicly financed programs of health care services or to such Member's race, color, sex, religion, place of residence (if there is reasonable access to Plan facilities), or socioeconomic status. This requirement does not apply to circumstances when Provider should not render services due to limitations arising from lack of training, experience, skill, or licensing restrictions. Plan shall provide Covered Services to Members in a manner consistent with the dignity of each Member as a human being.

9. **Member Eligibility Notification.** [WAC 284-170-421(1)&(6)(d); RCW 48.43.525(1); RCW 48.46.110(3)] In order to comply with Washington law, the following is added to Article 3 of the Agreement (KFHPWA's Responsibilities):

KFHPWA shall provide Provider with a means to obtain timely information on Member eligibility and benefits and with full access to Plan's coverage and service terms for a Member. KFHPWA shall not retrospectively deny coverage for emergency and nonemergency care that had prior authorization under the Plan's written policies at the time the care was rendered. Where Plan determines that a Member has received health care services to which such Member is not entitled under the terms of his or her Membership Agreement, Plan shall not have recourse against such Member for any amount above the actual cost of providing such service, if any, specified by such Membership Agreement, unless the Member or a member of his or her family has given or withheld information to Plan, the effect of which is to mislead or misinform Plan as to the Member's right to receive such services.

10. **Disputes.** [WAC 284-170-421(13); WAC 284-170-440; RCW 48.43.055]

- a. **Provider Complaints.** In order to comply with Washington law, the following is added to the provision regarding Provider appeals process set forth in Article 6 of the Agreement:

In accordance with RCW 48.43.055, KFHPWA provides an internal provider appeals process for provider complaints, including billing and payment issues, which are submitted in writing. If KFHPWA fails to grant or reject Provider's request submitted through the appeals process within thirty (30) days after the written request was submitted (or for billing disputes, within sixty (60) days after a written request was submitted), Provider may proceed as if the complaint had been rejected. A complaint that has been rejected may be submitted to non-binding mediation as mutually agreed by the parties.

- b. **Disputes Relating to Coverage Subject to the Jurisdiction of the State of Washington.** In order to comply with Washington law, the following is added to the provision regarding arbitration set forth in Article 6 of the Agreement:

Notwithstanding the foregoing, the parties agree that mandatory arbitration shall not apply to Disputes about Services provided to Members enrolled under Washington group or individual coverage issued by KFHPWA. KFHPWA acknowledges that, under Washington law, it is prohibited from requiring an alternative resolution process, including arbitration, to the exclusion of judicial remedies.

11. **Hold Harmless and Insolvency.** In order to comply with Washington law, the following is added to the provision regarding Member hold harmless set forth in Article 4 of the Agreement:

- a. **Hold Harmless.** [WAC 284-170-421(3)(a)] Provider hereby agrees that in no event, including, but not limited to nonpayment by KFHPWA, KFHPWA's insolvency, or breach of the Agreement will Provider bill, charge, collect a deposit from, seek compensation, remuneration, or reimbursement from, or have any recourse against Members or persons acting on their behalf, other than KFHPWA, for Services provided pursuant to this Agreement. This provision does not prohibit collection of deductibles, copayments, coinsurance, and/or payment for non-Covered Services, which have not otherwise been paid by a primary or secondary health carrier in accordance with regulatory standards for coordination of benefits, from Members in accordance with the terms of the Member's plan.
- b. **Insolvency.** [WAC 284-170-421(3)(b)] Provider agrees, in the event of KFHPWA's insolvency, to continue to provide the Services promised in the Agreement to Members of KFHPWA for the duration of the period for which premiums on behalf of the Member were paid to KFHPWA or until the Member's discharge from inpatient facilities, whichever time is greater.
- c. **Modification.** [WAC 284-170-421(2)&(3)(c)] Notwithstanding any other provision of the Agreement, nothing in the Agreement shall be construed to modify the rights and benefits contained in the Member's health plan. In the event of any conflict between the Agreement and a Member's health plan, the benefits, terms, and conditions of the health plan shall govern with respect to coverage provided to Members.
- d. **No Billing.** [WAC 284-170-421(3)(d)] Provider may not bill the Member for Covered Services (except for deductibles, copayments, or coinsurance) where KFHPWA denies payments because the Provider has failed to comply with the terms or conditions of the Agreement.

- e. **Survival.** [WAC 284-170-421(3)(e)] Provider further agrees (1) that Subsections (a), (b), (c) and (d) of this Section shall survive termination of the Agreement regardless of the cause giving rise to termination and shall be construed to be for the benefit of KFHPWA's Members, and (2) that this provision supersedes any oral or written contrary agreement now existing or hereafter entered into between Provider and Members or persons acting on their behalf.
- f. **Subcontracting.** [WAC 284-170-421(3)(f)] If Provider contracts with other providers or facilities who agree to provide Covered Services to Members of KFHPWA with the expectation of receiving payment directly or indirectly from Network or KFHPWA, such providers or facilities shall agree to abide by the provisions of Subsections (a) through (e) of this Section.

12. **Payment Collection.** [WAC 284-170-421(4)] In order to comply with Washington law, the following is added to the provision regarding Member hold harmless set forth in Article 4 of the Agreement:

Provider acknowledges that willfully collecting or attempting to collect an amount from a Member knowing that collection to be in violation of the Agreement constitutes a Class C felony under RCW 48.80.030(5) and (6).

13. **Overpayment Recovery-KFHPWA.** [RCW 48.43.600] In order to comply with Washington law, the following is added to Article 4 of the Agreement (Billing and Payment):

This Section applies only if Provider is a "health care provider" as defined in RCW 48.43.005(30).

- a. Except in the case of fraud, or as provided in Subsections (b) and (c) of this Section, KFHPWA may not: (1) request a refund from Provider of a payment previously made to satisfy a claim unless it does so in writing to Provider within twenty-four (24) months after the date that the payment was made; or (2) request that a contested refund be paid any sooner than six months after receipt of the request. Any such request shall specify why KFHPWA believes the provider owes the refund. If Provider fails to contest the request in writing to KFHPWA within thirty (30) days of its receipt, the request is deemed accepted, and the refund shall be paid.
- b. KFHPWA may not, if doing so for reasons related to coordination of benefits with another payer or entity responsible for payment of a claim: (1) request a refund from Provider of a payment previously made to satisfy a claim unless it does so in writing to Provider within thirty (30) months after the date that the payment was made; or (2) request that a contested refund be paid any sooner than six months after receipt of the request. Any such request shall specify why KFHPWA believes Provider owes the refund and include the name and mailing address of the entity that has primary responsibility for payment of the claim. If Provider fails to contest the request in writing to KFHPWA within thirty (30) days of its receipt, the request is deemed accepted, and the refund shall be paid.
- c. KFHPWA may at any time request a refund from Provider of a payment previously made to satisfy a claim if: (1) a third party, including a government entity, is found responsible for satisfaction of the claim as a consequence of liability imposed by law, such as tort liability; and (2) KFHPWA is unable to recover directly from the third party because the third party has either already paid or will pay Provider for the health services covered by the claim.

- d. If this Agreement or any other agreement between KFHPWA and Provider conflicts with this Section, this Section shall prevail. However, nothing in this Section prohibits Provider from choosing at any time to refund to a KFHPWA any payment previously made to satisfy a claim.
- e. For purposes of this Section, the term "refund" means the return, either directly or through an offset to a future claim, of some or all of a payment already received by Provider.
- f. This Section neither permits nor precludes KFHPWA from recovering from a Member or beneficiary any amounts paid to a health care provider for benefits to which the Member or beneficiary was not entitled under the terms and conditions of the health plan, insurance policy, or other benefit agreement.
- g. This Section does not apply to claims for health care services provided through dental-only health carriers, services provided under Title XVIII (Medicare) of the Social Security Act, or Medicare supplemental plans regulated under Chapter 48.66 RCW.

14. **Overpayment Recovery-Provider.** [RCW 48.43.605] In order to comply with Washington law, the following is added to Article 4 of the Agreement (Billing and Payment):

This Section applies only if Provider is a "health care provider" as defined in RCW 48.43.005(30).

- a. Except in the case of fraud, or as provided in Subsection (b) of this Section, Provider may not: (1) Request additional payment from Network or KFHPWA to satisfy a claim unless Provider does so in writing to KFHPWA within twenty-four (24) months after the date that the claim was denied or payment intended to satisfy the claim was made; or (2) request that the additional payment be made any sooner than six months after receipt of the request. Any such request shall specify why Provider believes KFHPWA owes the additional payment.
- b. Provider may not, if doing so for reasons related to coordination of benefits with another payer or entity responsible for payment of a claim: (1) Request additional payment from Network or KFHPWA to satisfy a claim unless Provider does so in writing to KFHPWA within thirty months after the date the claim was denied or payment intended to satisfy the claim was made; or (2) request that the additional payment be made any sooner than six (6) months after receipt of the request. Any such request shall specify why Provider believes KFHPWA owes the additional payment and include the name and mailing address of any entity that has disclaimed responsibility for payment of the claim.
- c. If this Agreement or any other agreement between KFHPWA and Provider conflicts with this Section, this Section shall prevail. However, nothing in this Section prohibits Network or KFHPWA from choosing at any time to make additional payments to Provider to satisfy a claim.
- d. This Section does not apply to claims for health care services provided through dental-only health carriers, services provided under Title XVIII (Medicare) of the Social Security Act, or Medicare supplemental plans regulated under Chapter 48.66 RCW.

15. **Provider/Patient Care.** [WAC 284-170-421(7); RCW 48.43.510(6) & (7)] In order to comply with Washington law, the following is added to the provision regarding communication with Members set forth in Article 8 of the Agreement:

KFHPWA may not in any way preclude or discourage its providers from informing patients of the care they require, including various treatment options, and whether in their view such care is consistent with medical necessity, medical appropriateness, or otherwise covered by the Member's plan with KFHPWA. KFHPWA shall not prohibit, discourage, or penalize a provider otherwise practicing in compliance with the law from advocating on behalf of a patient with KFHPWA. Nothing in this Section shall be construed to authorize providers to bind health plans to pay for any service. Furthermore, KFHPWA shall not preclude or discourage patients or those paying for their coverage from discussing the comparative merits of different health plans with their providers. This prohibition specifically includes prohibiting or limiting providers participating in those discussions even if critical of KFHPWA.

16. **Record Retention.** [WAC 284-170-421(8)] In order to comply with Washington law, the following is added to the provision regarding access to records set forth in Article 7 of the Agreement:

Provider shall make health records available to appropriate state and federal authorities involved in assessing the quality of care or investigating complaints, grievances, appeals, or review of any adverse benefit determination of Members subject to applicable state and federal laws related to the confidentiality of medical or health records. Provider shall also cooperate with audit reviews of encounter data in relation to the administration of health plan risk adjustment and reinsurance programs.

17. **Subcontractors.** [WAC 284-170-401; WAC 284-170-240] In order to comply with Washington law, the following is added to the provision regarding assignment, delegation, and subcontracting set forth in Article 2 of the Agreement:

If Provider subcontracts with other providers or facilities who agree to provide Covered Services to Members of KFHPWA, such subcontracts shall require each such provider or facility to abide by all requirements of WAC 284-170 Subchapter C. If Provider is permitted to delegate functions to be performed by it under this Agreement, Provider (a) shall include the applicable requirements of WAC 284-170 Subchapter B in its subcontract(s) addressing delegation, including providing the Washington State Office of the Insurance Commissioner with access to any pertinent information related to the subcontract during the contract term, for up to ten (10) years from the final date of the contract period, and in certain instances, where required by federal or state law, periods in excess of ten (10) years and (b) agrees that KFHPWA has the right to approve, suspend or terminate any such delegation.

18. **Coordination of Benefits.** [WAC 284-51; 284-51-215] In order to comply with Washington law, the following is added to the provision regarding coordination of benefits set forth in Article 4 of the Agreement:

The adjudication practices of KFHPWA pursuant to Section 4.6 (Coordination of Benefits) of the Agreement shall be administered to reflect WAC 284-51. KFHPWA shall not unreasonably delay payment of a claim by reason of the application of a coordination of benefits provision.

19. **Utilization Review.** [RCW 48.43.520; RCW 48.43.525; WAC 284-43-2000] In order to comply with Washington law, the following is added to Article 3 of the Agreement (KFHPWA's Responsibilities):

- a. KFHPWA shall maintain a documented utilization review program description and written clinical review criteria based on reasonable medical evidence. The program shall include a method for reviewing and updating criteria. KFHPWA shall make clinical protocols, medical management standards, and other review criteria available upon request to Provider. KFHPWA need not use medical evidence or standards in its utilization review of religious nonmedical treatment or religious nonmedical nursing care.
- b. The utilization review program shall meet accepted national certification standards such as those used by the National Committee for Quality Assurance except as otherwise required by Washington Administrative Code Chapter 284-43 and shall have staff who are properly qualified, trained, supervised, and supported by explicit written clinical review criteria and review procedures.
- c. When conducting utilization review KFHPWA shall follow all procedures and requirements specified in WAC 284-43-2000, including the following.
 - (1) For retrospective review, base review determinations solely on the medical information available to the provider at the time the health service was provided;
 - (2) Not retrospectively deny coverage for emergency and nonemergency care that had prior authorization under the plan's written policies at the time the care was rendered unless the prior authorization was based upon a material misrepresentation by the provider;
- d. KFHPWA shall reimburse reasonable costs of medical record duplication for reviews.
- e. KFHPWA shall have written procedures to assure that reviews and second opinions are conducted in a timely manner in accordance with the time frames specified in WAC 284-43-2000, including within thirty (30) calendar days for post service review requests.
- f. KFHPWA shall not penalize nor threaten Provider with a reduction in future payment or termination of Provider or participating provider status because Provider disputes KFHPWA's determination with respect to coverage or payment for health care services.

20. **Reporting.** [WAC 284-170-421(12)] In order to comply with Washington law, the following is added to Article 8 of the Agreement (Relationship of Parties):

KFHPWA shall not penalize Provider because the Provider, in good faith, reports to state or federal authorities any act or practice by the health plan that jeopardizes patient health or welfare or that may violate state or federal law.

21. **Prior Authorization.** [WAC 284-43-2050; WAC 284-43-2060] In order to comply with Washington law, the following is added to Article 3 of the Agreement (KFHPWA's Responsibilities):

- a. KFHPWA shall maintain a documented prior authorization program description and use evidence-based clinical review criteria. KFHPWA shall make determinations in accordance with the current clinical review criteria and use the medical necessity definition stated in the Member's plan. The program shall include a method for reviewing and updating criteria. KFHPWA need not use medical evidence or standards in its prior authorization of religious nonmedical treatment or religious nonmedical nursing care.

- b. The prior authorization program shall meet accepted national certification standards such as those used by the National Committee for Quality Assurance except as otherwise required by Washington Administrative Code Chapter 284-43 and shall have staff who are properly qualified, trained, supervised, and supported by explicit written clinical review criteria and review procedures.
- c. KFHPWA shall follow all prior authorization procedures and requirements specified in WAC 284-43-2050 and WAC 284-43-2060, including the following.
 - (1) KFHPWA shall have written policies and procedures to assure that prior authorization determinations are made within the appropriate time frames specified in WAC 284-43-2050. Time frames shall be appropriate to the severity of the Member condition and urgency of the need for treatment as documented in the prior authorization request.
 - (2) KFHPWA shall have an extenuating circumstances policy and procedure which eliminates the administrative requirement for a prior authorization of services when an extenuating circumstance prevents Provider from obtaining a required prior authorization before a service is delivered.

22. **Prescription Drug Utilization Management Exception Request Process.** [RCW 48.43.420; WAC 284-43-2020] In order to comply with Washington law, the following is added to Article 3 of the Agreement (KFHPWA's Responsibilities):

In accordance with WAC 284-43-2020, KFHPWA maintains clinical review criteria for prescription drugs as part of its drug utilization management program. The clinical review criteria and a readily accessible, convenient process to request an exception to the criteria are available on KFHPWA's website located at <https://wa-provider.kaiserpermanente.org>, or as Provider may be informed otherwise.

23. **Audio-Only Telemedicine.** [RCW 48.43.735; WAC 284-170-433] In order to comply with Washington law, the following is added to the provision regarding Member billing set forth in Article 4 of the Agreement:

To the extent audio-only telemedicine is a Covered Service, Provider's billing of a Member, and compensation to Provider hereunder, are conditioned upon: (i) Provider obtaining Member's written or verbal consent to such billing prior to such Covered Service being delivered, and (ii) beginning January 1, 2023, and as may be required by law, there exists an "established relationship" between Provider and such Member.

As defined in RCW 48.43.735(9)(d) "established relationship" means the provider providing audio-only telemedicine has access to sufficient health records to ensure safe, effective, and appropriate care services and: (i) The covered person has had, within the past three (3) years, at least one in-person appointment, or at least one real-time interactive appointment using both audio and video technology, with the provider providing audio-only telemedicine or with a provider employed at the same medical group, at the same clinic, or by the same integrated delivery system operated by a carrier licensed under chapter 48.44 or 48.46 RCW as the provider providing audio-only telemedicine; or (ii) The covered person was referred to the provider providing audio-only telemedicine by another provider who has had, within the past three (3) years, at least one in-person appointment, or at least one real-time interactive appointment using both audio and video technology, with the covered person and has provided relevant medical information to the provider providing audio-only telemedicine.

24. **Changes to Certain Documents, Procedures, or Administrative Policies; Material Amendments.** In order to comply with Washington law, the following is added to the provision regarding amendment set forth in Article 9 of the Agreement:

- a. **Changes to Documents, Procedures, or Administrative Policies that Affect Compensation or Health Care Service Delivery.** [WAC 284-170-421(6)] Provider shall be given reasonable notice of not less than sixty (60) days of changes to documents, procedures or administrative policies referenced in the Agreement that affect Provider's compensation or that affect health care service delivery unless changes to federal or state law or regulations make such advance notice impossible, in which case notice shall be provided as soon as possible. Subject to any termination and continuity of care provisions of the Agreement, a Provider may terminate this Agreement without penalty if Provider does not agree with the changes. No change to the Agreement may be made retroactive without the express written consent of the Provider.
- b. **Material Amendments.** [RCW 48.39.005; RCW 48.39.010]
 - (1) "Material amendment" means an amendment to a contract that would result in requiring Provider to participate in a health plan, product, or line of business with a lower fee schedule in order to continue to participate in a health plan, product, or line of business with a higher fee schedule.
 - (2) A material amendment does not include any of the following:
 - a. A decrease in payment or compensation resulting from a change in a fee schedule published by KFHPWA upon which the payment or compensation is based, and the date of applicability is clearly identified in the contract, compensation addendum, or fee schedule notice;
 - b. A decrease in payment or compensation that was anticipated under the terms of the contract, if the amount and date of applicability of the decrease is clearly identified in the contract; or
 - c. Changes unrelated to compensation so long as reasonable notice of not less than sixty (60) days is provided.
 - (3) KFHPWA shall provide no less than sixty (60) days' notice to Provider of any proposed material amendments to the Agreement.
 - (4) Any material amendment to the Agreement must be clearly defined in a notice to Provider as being a material change to the Agreement before Provider's notice period begins. The notice must also inform Provider that it may choose to reject the terms of the proposed material amendment through written or electronic means at any time during the notice period and that such rejection may not affect the terms of Provider's existing Agreement.
 - (5) Provider's rejection of the material amendment does not affect the terms of its existing Agreement.
 - (6) A failure to comply with Subsections (3), (4), and (5) of this Section shall void the effectiveness of the material amendment.